



HIGHER HEALTH

Higher Education & Training:
Health, Wellness and Development Centre



2022/23

ANNUAL REPORT



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INTRODUCING HIGHER HEALTH

HIGHER HEALTH, a non-profit organisation and an implementing agency of the Department of Higher Education and Training (DHET), is dedicated to promoting the health and wellbeing of about 1.7 million students in the post-school education and training (PSET) sector in South Africa.

It works in partnership with Universities South Africa (USAf), the representative body of public universities, the South African Public Colleges Organisation (SAPCO), the representative body of the technical and vocational education and training (TVET) colleges, and individual universities and colleges across the country.

Mission

To contribute to academic success among students in post-school education and training through improving and transforming their health, wellness and psychosocial development.

About this annual report

HIGHER HEALTH aims to provide to our wide range of partners and stakeholders a compact but comprehensive overview of our work and a sense of the organisation's progress over time. We appreciate that many individuals and organisations are directly engaged with a particular aspect of our programmes but may not be aware of the full range of interventions and the management of the organisation. We trust this report helps to contextualise your valuable contribution.

Our financial year runs from 1 April to 31 March, and our financial statements as well as matters related to governance and the national management of HIGHER HEALTH are reported for the period corresponding to the financial year.

However, our campus-based programmes are reported for the academic year that overlaps with most of the relevant financial year, in this instance January to December 2022. This is because we gear our programmes to the academic calendar and to the rhythms of student life.

Mandates

Mandate 1

Provide technical guidance and leadership in the post-school education and training (PSET) sector on the health, wellness and psychosocial wellbeing of students.

Mandate 2

Unify the sector response by coordination of stakeholders and role-players.

Mandate 3

Mobilise resources for the sector and for HIGHER HEALTH's activities and operations.

Mandate 4

Undertake sectoral research and the monitoring and evaluation of programme impact in the field of student health, wellness and psychosocial development.

Mandate 5

Develop skills and establish capacity building for student health, wellness and psychosocial development in PSET institutions.

Mandate 6

Conduct teaching and learning while overseeing the implementation of relevant service delivery programmes.

Mandate 7

Provide easy access to free health and psychosocial services to universities, TVET colleges, and community education and training (CET) colleges.

CHAIRPERSON'S MESSAGE



The period covered by this report was extraordinarily successful for HIGHER HEALTH, both in terms of campus programme delivery and the consolidation of the organisation.

The post-school education and training (PSET) sector only shook off the shadow of COVID-19 completely in 2022 and returned to full operation. This normalisation created conditions for HIGHER HEALTH to display its growing capacity for enabling campus-based health interventions. Readers will see evidence of this in the scale of delivery documented in this report and the level of student participation. In many instances, programme output not only equalled but exceeded that which was achieved in the pre-COVID period.

By the end of the reporting period, HIGHER HEALTH was on the brink of realising its vision of introducing an accredited curriculum that centres on health, wellness and the development of active citizenship. Through a partnership with the Health and Welfare Sector Education and Training Authority (HWSETA) and the Quality Council for Trades and Occupations (QCTO), HIGHER HEALTH has created a six-module certificate curriculum with NQF Level 5 accreditation. This is based on the Second Curriculum (now referred to the Co-Curriculum) introduced module by module over approximately five years as part of the HIGHER HEALTH model and the organisation's nurturing of active learning through peer education and student volunteerism.

All societies have a need to build civic activism and South Africa – which is still grappling with the bitter legacy of apartheid – has a greater need than most. There were many possible paths to incorporating this type of education in university and college life and it is to HIGHER HEALTH's great credit that it was first to seize the initiative.

The organisation is supported financially by local and international funding partners in the health and higher education and training sectors and has been able to grow and diversify its funding base as its vision has expanded. Our Chief Executive Officer, Prof Ramneek Ahluwalia, has a keen understanding of the organisational environment, appreciates which organisations have common interests with HIGHER HEALTH, and recognises mutually satisfying partnerships can be forged. The organisation's financial sustainability owes much to this leadership.

HIGHER HEALTH's funding for the year and funding commitments totalled some R142 million, an amount that demands strong financial management capability and close oversight. Adjustments to our financial

management systems are made from time to time to keep pace with budgetary growth. I am pleased to say that we received a clean audit opinion for the 2022/23 financial year.

In 2022, we marked the fifth anniversary of HIGHER HEALTH's incorporation as a not-for-profit company and reviewed the strategy which had served us well for the first five years. The new strategy involves fine-tuning rather than a radical change. It completes the journey away from an HIV-focused organisation to a broad health promotion and developmental entity. It more fully reflects the uniqueness of the PSET sector and it specifies the organisation's mandate within this sector more closely.

During the year, the organisation experienced a fair degree of change in the composition of the board and in the ranks of senior management. While this inevitably presented some challenges, HIGHER HEALTH was able to manage the transitions effectively and without disruption to our partners at campus level.

All in all, the performance recorded in this report is a tribute to collaboration. We are indebted to the Minister of Higher Education, Science and Innovation, Dr Blade Nzimande, and Deputy Minister Buti Manamela for their unflagging support and keen appreciation of the value that HIGHER HEALTH brings to the sector.

Our ability to deliver is also testimony to the commitment of our partners – funders, the management, students and staff of universities and colleges, the Departments of Health and Higher Education and Training, provincial and district health services, other public entities, technical experts, training authorities, and a range of non-governmental organisations.

I wish to recognise the dedication of members of the HIGHER HEALTH Board who continue to provide guidance and counsel, and the outstanding leadership of Prof Ahluwalia and his very solid team across the country. We hope to continue exploring the intersection between education and wellness – and their joint contribution to a more civil society – for many years to come.

Prof Wim De Villiers
Chairperson

DEPUTY CHAIRPERSON'S MESSAGE



I am proud to introduce this report on a bumper year for HIGHER HEALTH, a period when COVID-19 receded, our campuses returned to normal life, and student health and wellness programmes were appreciated as never before.

As head of a technical and vocational education and training (TVET) college, I appreciate how constrained resources in our sub-sector often are. We may want to enrich the campus years that bridge childhood and adulthood with support services and extramural activities, but often the means are lacking. In this context, HIGHER HEALTH has been a gift of great and varied value, including:

The value of leadership: HIGHER HEALTH's emphasis on peer-to-peer health promotion has given thousands of young students an opportunity for personal growth through service to their fellow students. TVET college courses tend to be quite technical and may not address soft skills like building self-confidence and teamwork. HIGHER HEALTH provides opportunities precisely for this kind of growth among our students. By making a firm connection between health activism and broader civic engagement, HIGHER HEALTH has taken its commitment to youth empowerment up a few more notches.

The value of safe spaces: South Africa is undeniably a complex and troubled society and many of our students are vulnerable – vulnerable to specific threats like gender-based violence, mental health disorders, the use of damaging substances or, more generally, to the draining effects of poverty. HIGHER HEALTH has begun to create places of refuge for students in difficulty – services they can approach and adults or peers they can turn to. There is still a long way to go in developing these, but a start has been made.

The value of expertise: College management and staff have benefited immeasurably from HIGHER HEALTH's ability to access health experts and produce policies, guidelines and protocols so that we have the competence to address critical health issues. The COVID-19 pandemic is the outstanding example, but close behind is GBV, sadly an ever-present threat to our woman students. In helping us take charge of our responses to these matters, HIGHER HEALTH has strengthened our institutions.

So, to the funders who provide resources for HIGHER HEALTH to bring its dynamic vision of health and wellness to life and to the management and staff HIGHER HEALTH who are our tireless partners, our heartfelt thanks. We are particularly grateful to the campus health and wellness coordinators (CHWCs) seconded by HIGHER HEALTH to our colleges, and to Prof Ahluwalia for his unique leadership.

Mrs Dipiloane Phutsisi
Deputy Chairperson



SECTION 1:

CEO'S OVERVIEW

CEO'S OVERVIEW



The reporting year was a standout period of growth for HIGHER HEALTH and this is testimony to the trust of funders who have invested in our work, the diligence of our staff and partners on the ground, and the receptiveness of students to health and wellness programmes.

- Not only did our health and wellness activities reach many more students in 2022/23 than previously (see box on right) but *we have truly embedded a comprehensive approach to youth health and wellness*. COVID-19 served as a catalyst for this as mental health issues and gender-based violence (GBV) became more acute during this period and demanded immediate action. The mental health and GBV services which we established then have been sustained and are in demand. We have focused on building the capacity of universities and colleges to implement a multi-faceted response to GBV with a strong emphasis on prevention. Our original focus on HIV, TB and sexual and reproductive health (SRH) remains as relevant as ever.
- Our established knowledge transfer practices and peer-to-peer methodology combined to give birth this year to *a unique new product: our Civic Education and Health Skills Programme*. This is an online certificate course with a strong practical component that has been accredited by the Quality Council for Trades and Occupations (QCTO). Our pioneering pairing of health skills and civic education grows from our experience that personal empowerment is a vital ingredient of good health decisions – and health activism can be a viable route to civic engagement. Read more about this on page 10.
- The above progress was made possible by the confidence that the Department of Higher Education and Training (DHET) and a group of funders and partners have displayed towards us. We are conscious that our reputation is intertwined with yours and that the burden of good management and governance grows as responsibility and resources increase. We are pleased to report that our audit report for the year is unqualified. Furthermore, we have satisfied the requirements of major audit firms acting on behalf of various funders.
- During the year, HIGHER HEALTH had the honour of representing the department and the post-school education and training (PSET) sector at significant forums both domestically and abroad. This speaks to the quality of our relationships and to the innovative programme you have had the courage to co-create with us.

Serving more students on more campuses

HIGHER HEALTH worked with staff and students at **108 university campuses**, **274 TVET college campuses** and **168 CET centres**.

▶ Knowledge transfer

Total attendance at Co-Curriculum sessions exceeded **725 000**. Topics: HIV and sexual and reproductive health, mental health, GBV, alcohol misuse, and rights of LGBTQI+ students and those with disabilities.

More than **685 000 students** benefited from peer-to-peer awareness campaigns and activities.

Up to **785 000 listeners** a week tuned in to health discussions on **20 campus** and community radio stations.

▶ Student actions for health

About **5 300 students** and **staff** signed up as peer educators, mobilisers and mentors or coordinators.

A group of **90 students** participated in the Inaugural Mandela-Sobukwe Leadership Retreat.

Students completed more than **326 000** self-assessments for various health risks.

A total of **130 342** students sought an HIV test, **184 762** underwent TB screening, **201 105** were screened for STIs, and **217 843** for diabetes, hypertension and high cholesterol.

Nearly **12 000** phoned or messaged the 24-hour crisis helpline for psychosocial help.

▶ Health products and services supplied

- **6.45 million** condoms
- **114 377** contraceptive products
- **15 000** referrals for psychosocial services
- **5 308** referrals for GBV-related psychosocial care
- **11 185** counselling sessions by HIGHER HEALTH psychologists

At all times HIGHER HEALTH has respected the primacy of the mandate of the PSET sector to deliver teaching and learning, research and innovation, and community service. We have seen our role as supporting this mandate and we believe the nature of our support is evolving and becoming stronger. The thousands of peer educators the programme recruits, trains and deploys each year have provided meaningful service to campus communities. Our health education interventions have matured to the point of generating our accredited Civic Education and Health Skills course. We strive to ensure our health programmes are evidence-informed and that critical reflection on our practices leads to innovation.

As HIGHER HEALTH has become clearer about the essence of our work, we have grown more flexible about the way it is implemented. Circumstances at universities, technical and vocational education and training (TVET) colleges, and community education and training (CET) colleges vary considerably. Some of our methods were designed mainly with TVET colleges in mind and this constrained delivery at universities. As universities returned to full-scale contact teaching and learning in 2022, they introduced variations to the health and wellness programme delivery and the results were impressive. Performance at universities took off on all fronts and this dynamic boosted our sector-wide outputs to new heights, as indicated in Section 3 of this report.

Mobilisation for health and wellness

HIGHER HEALTH focuses on aspects of health and psychosocial wellness that are of greatest concern to young adults. These are SRH, including HIV and other STIs, mental health, including the stresses experienced by many LGBTQI+ students and students with disabilities, GBV and substance misuse.

Failure to assist students to manage the health challenges confronting them can undermine their academic performance and contribute to them abandoning their studies – and the hope of a career that requires a particular qualification.

HIGHER HEALTH places great emphasis on preventive health strategies that were implemented in 2022 by 5 000-plus campus health and wellness coordinators (CHWCs), peer mentors, and peer educators and mobilisers. They are responsible for knowledge transfer on health and wellness via campaign style interventions under the banner of First Things First, and through more structured educational sessions constituting the Co-Curriculum. They also facilitate students' self-assessments, using tools which help students appreciate whether their health is at risk and whether they need to seek professional assistance.

The power of the peer-to-peer education system to generate student participation in health and wellness activities is reflected in graphs on pages 7 and 8.

While the number of First Things First health days on campus remained roughly the same in 2022 as the previous year, the attendance at Co-Curriculum sessions improved dramatically for all areas of health, as did the number of students completing self-assessments of their levels of health risk.

In the area of HIV, TB, STIs and SRH, this participation carried through into increased demand for HIV testing and screening for TB and STIs.

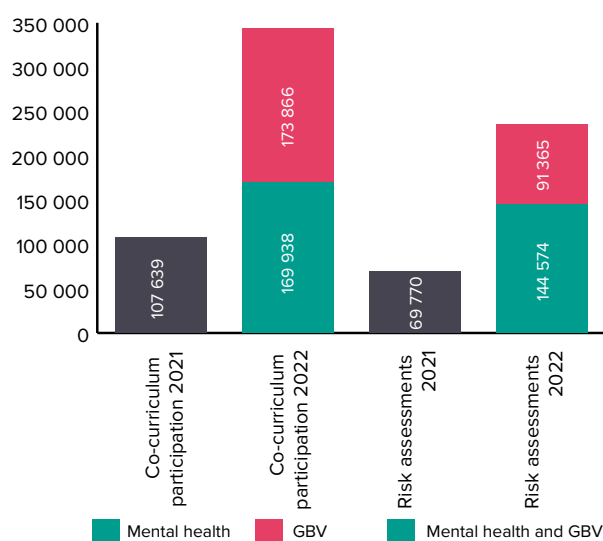
These activities identified a small percentage of students who were at risk of one or more health condition and most of these students were linked to an appropriate service. Equally importantly, health days, the Co-Curriculum and risk assessment are designed to sensitise all participants to the need to take action to sustain their good health. We encourage students to be proactive in using preventive products, initiating lifestyle changes, and visiting health services as an early option rather than a last resort when they have a problem.

Participation in prevention activities: HIV, TB, STIs and SRH



CEO's overview *continued*

Participation in prevention activities: mental health and GBV



Note: Mental health and GBV constituted one module in 2021 but were split into two modules in 2022. Many students would have done both modules in 2022. Figures reflect cumulative attendance at module sessions. Individual students could be counted twice.

Access to health products and services

It is challenging to maintain a balance between raising awareness and generating a demand for health services and products, on the one hand, and delivering the needed services and products, on the other.

The scale on which campus health programmes supply different health products and provide services varies from millions to a few hundred each year. At the top end of the supply scale, campuses distributed about 6.54 million condoms in 2022 but, in contrast, only facilitated access to pre-exposure prophylaxis (PrEP) – a daily antiretroviral pill to prevent HIV – in 12 036 instances.

Often the linkage of students to services relies on referral to a health facility in the community as a minority of campuses have an on-site clinic and HIGHER HEALTH's mobile clinics have a finite reach. Even where on-campus facilities do exist they may not offer the full range of services students need. For example, they may not stock TB drugs or antiretrovirals (ARVs) or be accredited to provide antiretroviral treatment (ART).

Overall, the figures reported in detail in Section 3 of this report show an improvement in access to services but also tell us we have a long way to go.

For example, the uptake of contraception by students increased by 77% in 2022 but still only achieved a total output of 114 377 products. With more than a million

women students in the sector, it is clear we are reaching only a fraction of the need for contraception. The fact that 28 063 students requested pregnancy tests and 956 were referred for termination services only underscores the importance of strengthening access to SRH services.

The USAf/SAMRC national mental health survey also gave us an appreciation of the potential benefits of scaling up psychosocial care (see page 9). It found that nearly one in five university students requires some kind of psychosocial support – that is, about 220 000 students in the university sub-sector alone, before adding college students. HIGHER HEALTH has done valuable work in the sector in breaking down stigma concerning mental illness, creating a toll-free crisis helpline in partnership with the South African Depression and Anxiety Group (SADAG), and employing several clinical psychologists. The support we provide is significant: in 2022 about 12 000 students contacted the helpline, nearly 15 000 students from across the PSET sector were referred to psychosocial services, and our own psychologists conducted 11 185 counselling sessions. But clearly we need to do more and be innovative in order to approach the required scale of care.

HIGHER HEALTH considers that both demand and supply side factors constrain student access to services and utilisation of health products. Progress in both areas will require perseverance.

On the demand side, we need to consider that HIGHER HEALTH works in sensitive areas of healthcare – SRH, GBV and mental health – and overcoming barriers to health-seeking behaviour is bound to take time. We have got to the point where hundreds of thousands of students are willing to discuss these health concerns and admit they could be affected, but many may still be hesitant to commit to decisive action. Our job is to continue to create a more conducive environment for students to make and implement smart health decisions.

On the supply side, we are not always able to offer students convenient on-campus access to youth-friendly health services – and easy access to products such as PrEP, ART and contraception.

The quality of care at public health clinics is variable. Where clinic staff are over-burdened and stocks of medication and health consumables unreliable, HIGHER HEALTH's ideal of linkage to youth-friendly care is unlikely to be realised. We also appreciate that the public health sector is still finding its way back from the trauma of COVID-19 and has its own strategic priorities, such as preparation for the introduction of National Health Insurance. In districts supported by Global Fund grants, we are fortunate to have partner organisations that are funded to help strengthen public health clinics.

The reality is that no single entity can deliver comprehensive healthcare services to the student population of South Africa. The task demands well-structured partnership arrangements in which various parties make a defined contribution. During

2022 HIGHER HEALTH began to engage provincial health authorities with a view to concluding formal memorandums of understanding with them. We will accord the highest priority to continuing this process in the year ahead.



STUDENT MENTAL HEALTH: resilience and frailties revealed

Most South African university students believe they enjoy excellent or good mental health, but about one in five would benefit from psychosocial support, and a small sub-group needs specialised clinical services.

This a key finding of a massive survey on the mental health of South African university students, preliminary results of which were shared with university management in a webinar in April 2022. The study has since been published in the *Journal of Affective Disorders*.

It was initiated by Universities South Africa (USAf), funded by the South African Medical Research Council (SAMRC), and forms part of the World Health Organisation's World Mental Health International College Student Initiative. A sample of 29 000 students at 19 public universities participated in the survey.

In his virtual briefing to university managers, lead researcher Prof Jason Bantjes indicated that 14% of students who considered themselves to be in fair mental health required low-intensity psychosocial support and 4% who felt their mental health was poor needed more specialised services.

The survey attempted to assess mental health disorders experienced by respondents in the 30 days prior to the survey. It found the prevalence of major depressive disorders to be 15.4%, generalised anxiety disorder 10.9%, panic disorder 7.2% and bipolar disorder 1.8%. About 21% of students experienced attention difficulties, 5.1% abused drugs and 2.5% were dependent on alcohol. Suicidal behaviour was high: 8.6% of students said they had made a suicide plan and 2.3% had attempted suicide.

The survey was conducted during the COVID-19 pandemic but comparison with data collected pre-pandemic at the University of Cape Town and Stellenbosch University indicate that depressive and anxiety disorders were already high before COVID.

Prof Bantjes reported that the prevalence of mental health disorders was unevenly spread across the student population:

- Anxiety disorders were consistently higher at historically white institutions.
- Younger students were more likely to experience psychological distress.
- Black students at historically white institutions were at higher risk of mental disorders than white students.
- Risk of mental illness was also higher among gender non-conforming students, sexual minorities and students whose parents had a low level of education.

He commented that the survey did not explain the higher vulnerability of black students at historically white universities but it flagged the possibility that "the culture of these institutions and levels of alienation and marginalisation" played a part.

A concerning finding was that 70% of students with mental disorders were not getting help. The main reasons for this were: the limited availability of affordable services, reluctance to seek professional help, stigma attached to mental illness, doubts about the effectiveness of therapy, and the individual's lack of awareness that help was necessary.

Prof Bantjes said a range of interventions would be necessary to respond to the situation revealed by the survey. These included some interventions already initiated by HIGHER HEALTH: addressing stigma, providing psychoeducation to enable help-seeking, and peer-to-peer interventions. He suggested that receiving help via the internet might prove acceptable to students and effective in some cases.

Source: J Greenleaf Walker. *Towards improving the mental health of students at South African universities*. www.usaf.ac.za

CEO's overview *continued*

From Co-Curriculum to NQF Level 5 course

The past year saw a breakthrough for HIGHER HEALTH after years of wrestling with the question of how to leverage the power of a formal curriculum to build health knowledge and health promotion skills in the PSET sector. Various options have been tried and abandoned over the course of two decades.

Finally, last year, the National HIGHER HEALTH Civic Education and Health Skills Programme was accredited by the QCTO as an NQF level 5 qualification. The programme is based on the modules constituting the Co-Curriculum but adds substantial new content on civic responsibility and leadership.

- Any student registered at a PSET institution is eligible to enrol in the programme.
- The programme is delivered on a digital platform that can be accessed through various devices including mobile phones.
- Each of the six modules has a major practical element.
- Participants may fulfil their practical requirements by volunteering as peer educators on their campuses and in their communities.

I believe that this approach to a health skills curriculum can and will succeed for a host of reasons.

- It is embedded in a well-established health programme that has roots in every public university, TVET college and all provincial CET colleges.

- It runs alongside the academic, professional or vocational course that the student is enrolled in and does not interfere with that course.
- It builds complementary “soft skills” and resilience that help students thrive on campus and enhance their readiness for work at the end of their degree or diploma.
- It promotes civic values that are compatible with the transformation strategies of many PSET institutions and may help address social issues that emerges from time to time on some campuses.
- Students leave with a nationally accredited certificate in return for their service to the campus or community, an asset that may give them the edge in a difficult job market.
- A formal, practice-based civics course is a relevant response to the national debate on how to strengthen democracy and public sector accountability.

The design of the programme took HIGHER HEALTH's staff into uncharted educational waters and we depended heavily on the advice and guidance of experts in this field. I would like to recognise and extended my deepest gratitude to staff members at the HWSETA and QCTO who steered us along the correct path and to Dr Edna Rooth for her expert assistance.

The National Civic Education and Health Skills Programme will be offered to students within a pilot project in the second half of 2023 and will be taken to scale early in 2024.

Value proposition for National Civic Education and Health Skills Programme

Builds good citizenship and civic education skills in next generation workforce

Is cost-effective and easy to access on mobile phone. Builds digital skills

Builds soft skills that complement hard skills of next generation workforce

Adds an NQF Level 5 certificate to CV of young work seekers

Develops young minds for social entrepreneurship

Provides community education through 'Each 1 Teach 10' practical requirement

Strengthens learning in first year of PSET which is key to later academic success

Offers a quality-assured skills programme for relevant PSET staff

COVID-19: not quite gone and not forgotten

With the benefits of vaccination for COVID-19 and milder variants of the SARS-CoV-2 virus, the PSET sector commenced a full-scale return to campus in early 2022. The mood was cautious. Vaccine mandates were enforced at many institutions and infection control measures were still in place. A sharp spike in infections in early autumn provided proof that this watchful approach remained necessary.

HIGHER HEALTH continued to provide extensive training to staff and student volunteers to ensure institutions were able to follow the relevant infection control guidelines and protocols. This was particularly necessary at CET centres that had not had the benefit of earlier training.

As government modified various regulations and eventually lifted the COVID-19 state of disaster, we ceased to focus specifically on COVID-19. But we cannot afford to lose the primary lesson of this pandemic: the need to maintain vigilance and preparedness for future outbreaks of infectious diseases.

Public health experts warn that infectious pandemics will become more frequent occurrences. In addition to such global events, universities and colleges – with their classrooms and residences – are also vulnerable to localised outbreaks. It is not inconceivable that cholera or meningitis, for example, could impact campuses.

HIGHER HEALTH will continue to refresh the knowledge of our campus health and wellness coordinators (CHCWs) and health programme coordinators on the basic processes for managing outbreaks.



Partnerships that anchor our work

HIGHER HEALTH is a venture realised entirely through a complex array of partnerships. Our funding, most of our human resources, and many of the products and services needed to fulfil our mission are contributed by partners who share our vision for the wellbeing of young, bright, ambitious South Africans. Much of our energy and inspiration also derives from these dynamic relationships.

The primary partnerships, upon which the rest depend, are with **the institutions comprising the PSET sector: universities, TVET colleges, and CET colleges and centres**. Some of these partnerships were established many years ago, while others have been formed recently. I want to believe that they have been strengthened in recent years as HIGHER HEALTH's holistic approach to health made us more relevant to institutions and as COVID-19 cruelly drove home the realisation that health security is a prerequisite for a functioning society.

The partnerships with institutions are secured by multiple relationships at various levels – with top management, with other managers in areas as diverse as transformation and security, with student leaders and volunteers, with academics and particularly with health programme coordinators and health service providers.

Next most critical are our partnerships with the **Ministry and Department of Higher Education and Training (DHET)** and with a range of other **funding organisations**.

The delegation of some key responsibilities for health in the PSET sector to HIGHER HEALTH by Minister of Higher Education, Science and Innovation Dr Blade Nzimande served to cement HIGHER HEALTH's position in the sector. The confidence of the Minister is an invaluable asset and we have been privileged that aspects of our work are almost as an extension of the DHET – for example, the COVID-19 response and work to combat GBV in universities and colleges.

HIGHER HEALTH receives substantial **funding from the DHET and from the National Skills Fund (NSF)**, including support for our mobile clinics. This heightens our sense of accountability to the entire PSET sector and to the tax-paying South African public.

The **Global Fund to Fight HIV, TB and Malaria** has maintained a strong focus on youth around the world and HIGHER HEALTH has had the good fortune to receive several consecutive grants from the fund. There is a strong convergence in our approaches to HIV – with an emphasis on the social and structural drivers of this epidemic – and we have benefited from the Fund's rigorous approach to monitoring and the training it provides for its implementing partners.

CEO's overview *continued*

The **European Union** funded the development of critical tools during the COVID-19 pandemic and continues to support HIGHER HEALTH.

Among our most valuable implementing partners are the national **Department of Health, provincial departments of health, many health districts, and individual health facilities** in communities where PSET institutions are located. Their collaboration and provision of critical health consumables has been fundamental to the expansion of our services to students.

The **South African Depression and Anxiety Group (SADAG)** is the operational force behind our helpline and we are grateful to them for three significant years of collaboration. The ongoing guidance of members of our GBV Technical Task Team has been critical to progress made in this area.

Sustainable funding and financial management

HIGHER HEALTH's substantial income derives entirely from grants and donations from domestic public entities and international organisations, and from interest earned on such income.

Our most significant and consistent funders are the DHET and the NSF. In the case of the DHET the grant is confirmed through Treasury's national budgetary process and informed by the three-year perspective of the medium-term expenditure framework (MTEF). This plays a significant role in stabilising our finances through core organisational funding.

Other grants are tied to specific aspects of our work. For example, Global Fund investments are dedicated to youth health programmes with a strong HIV focus in specific sub-districts. Various grants and donations that were made for the COVID-19 response have now come to an end.

The consistent pattern has been a steady growth in income and an increase in the share of income derived domestically.

Summary of income and expenditure

	2023 R	2022 (Restated) R
Revenue and other operating income	94 220 026	79 892 157
Operating expenses and losses	97 633 473	77 219 650
Operating (loss) or profit	(3 413 477)	2 672 507
Finance costs	(172 389)	(271 974)
(Loss) or profit for the year	(3 585 836)	2 400 533

Revenue and other income amounted to R94.22m in 2023, an increase of 18% on the previous year. The growth in operating expenses amounted to 26%, outpacing the growth in revenue and HIGHER HEALTH recorded a R3.85m deficit in 2023.

However, at the end of the financial year, we also had deferred income amounting to nearly R48.32m, which comprised grant funding payable in terms of contractual arrangements with funders.

In terms of expenditure, the three largest items were employee costs of R45.20m (2022: R34.99m), consulting fees of R16.36m (2022: R14.44m) and travel expenses of R13.56m (2022: R7.6m). Collectively these items accounted for 77% of operating expenses.

We are pleased that we were given a clean audit opinion for the 2022/23 financial year.

Strengthening core management systems

HIGHER HEALTH continues to grow, both in terms of the finances under our management and the human resources that allow us to fulfil our mandate. This inevitably requires strengthening of our management capacity and systems.

At the end of the year, we had 178 employees. With 46% of our spending allocated to the cost of employees they are our greatest expense and our greatest asset. Our programmes are human-centred and our employees are without question the key to our success. In 2023 we prioritised the strengthening of our human resources function, realising that the scale of our organisation required more robust systems and procedures. More detail of this appears on page 38.

Monitoring and evaluation constituted another priority area for the year as it is key to determining whether we are meeting our strategic objectives and delivering in accordance with our funding agreements. At the start of the 2022/23 financial year HIGHER HEALTH appointed an M&E lead and this put our feet firmly on the path to building a robust monitoring, evaluation and learning (MEL) system.

Over the year, an M&E framework was developed – which links programme objectives to key performance indicators and these, in turn, to specific targets – along with tools and protocols to enable our provincial offices to gather data in a consistent and accurate manner.

We invested in an appropriate operating system and software which allows head office to obtain a real-time picture of the situation in all provinces.



We conducted data quality assessments in eight provinces and the results were mixed. Most provinces' scores were medium to good across seven parameters, but three provinces performed poorly on a few measures. The process enabled HIGHER HEALTH to pinpoint weaknesses and address them specifically.

Acknowledgments

It is an enormous honour to have the trust and goodwill of the many role-players in the PSET sector who have made possible the HIGHER HEALTH adventure in pursuit of student health and wellness. To each and every one of you who has made a contribution, my most sincere thanks.

While I cannot acknowledge every contribution individually, I would like to thank our major supporters.

We are grateful to the Minister of Higher Education, Science and Innovation, Dr Nzimande, for the encouragement he has given HIGHER HEALTH and the responsibility he has entrusted to us, and to Deputy Minister Buti Manamela for his sustained support over many years.

We appreciate the leadership of vice-chancellors of universities and principals of TVET and CET colleges in recognising and endorsing our health endeavours and to diverse members of management and staff for their energy and collaboration. We are also grateful for the facilitating role played by USAf and SAPCO in working with their constituencies.

We would like to recognise the role of student leadership, peer educators, and campus and community radio stations in mobilising campuses in support of health and wellness.

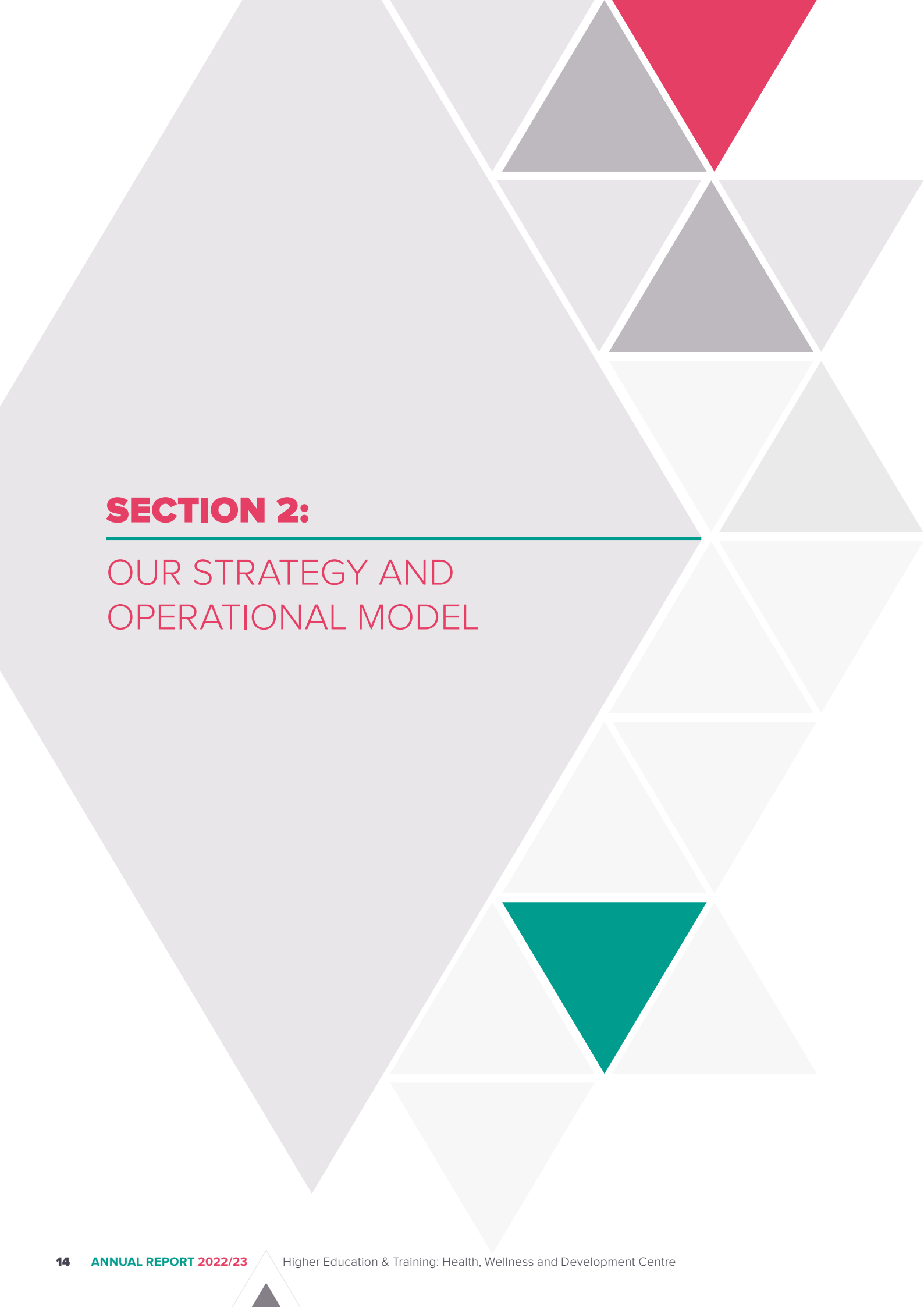
We are deeply appreciative of our funders for the resources they have entrusted to us and for the confidence in our work that this represents. HIGHER HEALTH has been innovative and has not always adhered to established programme formulas – and we are grateful for funders who are prepared to invest in new ways of working.

The National HIGHER HEALTH Civic Education and Health Skills Programme is perhaps our most original move to date and we owe a debt of gratitude to all who helped us achieve it.

At a service-delivery level, campuses depend to a large extent on partnerships with various provincial and district health services as well as non-governmental and community-based organisations. We would like to recognise their invaluable contribution.

Finally, I wish to acknowledge the constructive guidance provided by members of the HIGHER HEALTH Board whose responsibilities grow as our work expands, the tireless efforts of our growing body of employees across the country, and the value added by those who play an advisory role to complement the expertise of our staff members. The fact that this has been an outstanding year for HIGHER HEALTH is largely due to the commitment and resilience of our team of employees – at all levels of the organisation – and their ability to work collaboratively with our many partners.

Dr (Prof) Ramneek Ahluwalia
Chief Executive Officer



SECTION 2:

OUR STRATEGY AND OPERATIONAL MODEL

OUR STRATEGY AND OPERATIONAL MODEL

HIGHER HEALTH delivers health and wellness programmes within the unique environment of the post-school education and training (PSET) sector and therefore our goals are firmly tied to the educational goal of producing well-qualified, successful graduates. This not only entails preventing or alleviating the physical and psychosocial conditions that often undermine students' ability to focus on their studies. It also involves building health awareness, self-care skills and a sense of civic responsibility that complement the academic curriculum, producing graduates with a wider range of competencies relevant to the workplace, family life and participation in the broader community.

Strategic Plan 2022 – 2027

HIGHER HEALTH developed a new five-year strategic plan in 2022. The strategy represents continuity rather than a major change in direction. In many ways it formalises and elevates innovations that were introduced in the period following the launch of the previous strategic plan.

The mission and mandates of HIGHER HEALTH can be found on page 2 and the goals of the new strategy are set out below, to give readers a flavour of the new plan.



Goal 1

Through health awareness-creation, improve the knowledge of students in order to increase health-seeking behaviour which may contribute to timely completion of their studies.



Goal 2

Provide capacity building and skills training on health, wellness and psychosocial policy guidelines for staff at HIGHER HEALTH and PSET institutions.



Goal 3

Provide accessible, quality health and psychosocial support services to students across the PSET sector.



Goal 4

Expand HIGHER HEALTH's teaching and learning through extramural Co-Curriculum courses that build health and wellness knowledge and skills of students.



Goal 5

Contribute to the development of an institutional environment that respects human rights and promotes responsible citizenship as a foundation for student health, wellness and psychosocial development.



Goal 6

Maintain HIGHER HEALTH's funding base through a sustainable revenue model.



Goal 7

Strengthen and expand partnerships with institutions in all PSET sub-sectors, relevant government departments, funding and donor agencies, and other stakeholders in the broad field of student health, wellness and psychosocial development.

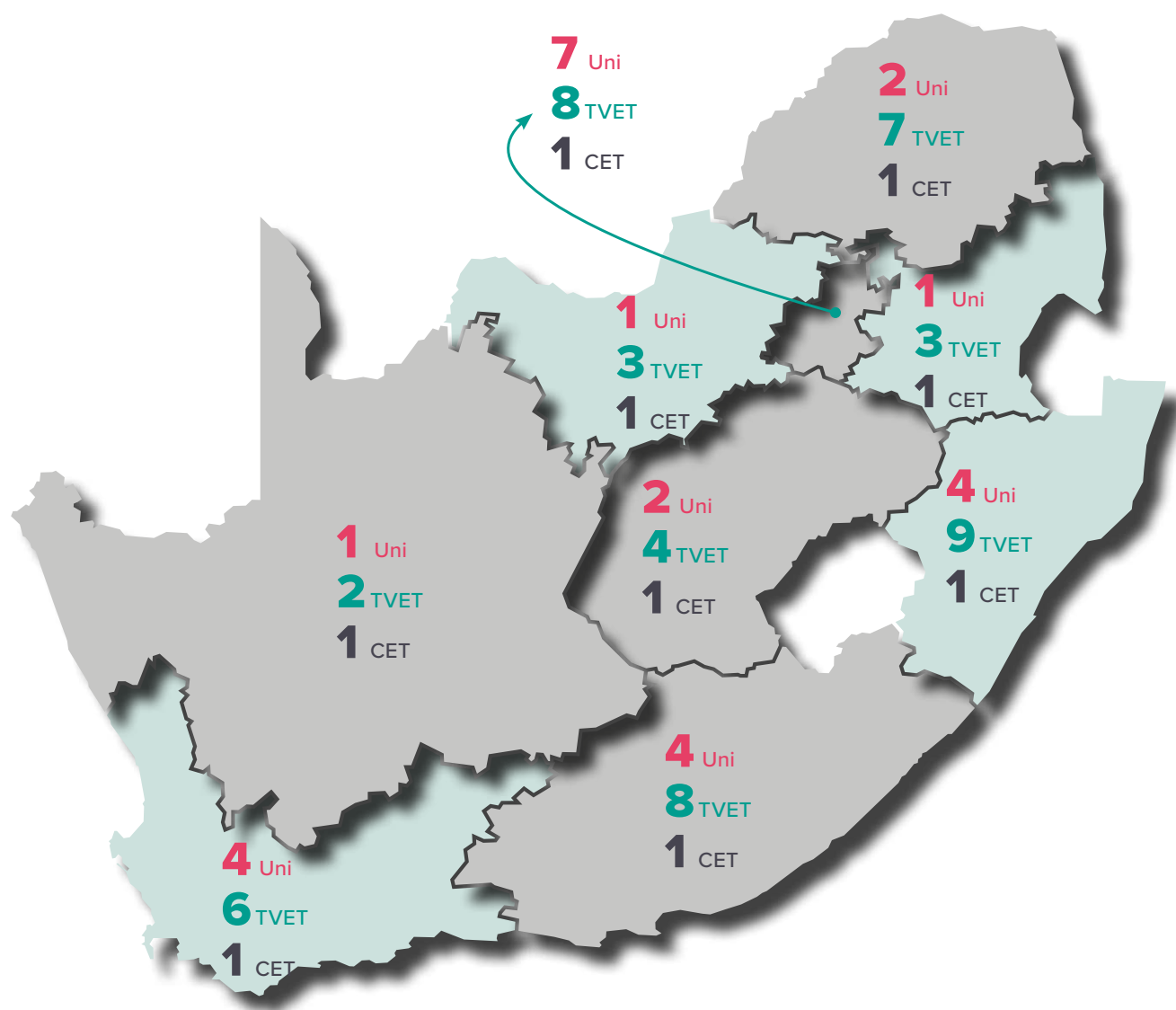


Goal 8

Promote the use of social and traditional media, and digital communication to create awareness of the social and structural determinants of health.

Our strategy and operational model *continued*

The post-school education and training landscape



	Public universities	TVET colleges	CET colleges
Institutions	26	50	9
Campuses reached by HH	108	254	3 276*
Students	1 094 808	452 277**	142 538

Notes:

* As per notice in Government Gazette. However, DHET notes current data on CET centres is not available

** TVET college enrolment dropped 33% in 2020 because two intakes were forfeited due to Covid-19. Enrolment has recovered but is still well below pre-pandemic levels

Source Statistics on post-school education in South Africa 2020. DHET, Pretoria, 2022

The HIGHER HEALTH Model

For several years, HIGHER HEALTH's support to students on university and college campuses has been informed by a model that comprises three levels of intervention.

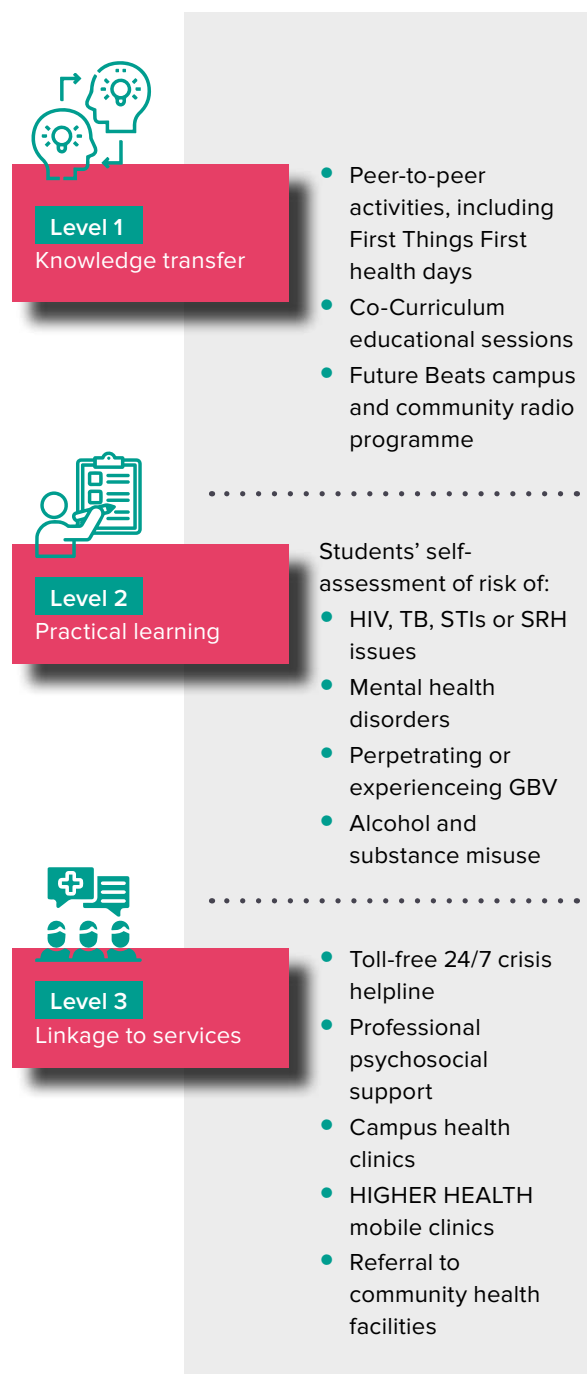
Level 1 Knowledge transfer: This acknowledges that students require sound information in order to take measures to protect their health and wellbeing. Knowledge transfer features awareness-raising activities and events undertaken by student volunteers and peer educators, as well as more structured health education modules delivered through the extramural Co-Curriculum (referred in the previous reports as the Second Curriculum). A third element is the Future Beats radio programme which enables campus and relevant community stations to carry regular health content and encourages debate on the social and structural factors that impact on student health.

Level 2 Practical learning: This flows from knowledge transfer and involves students applying their new understanding of an aspect of health to their own circumstances and behaviour. They do this by means of a self-administered assessment which indicates whether they are at risk of illness or harm. The system ensures that students who are at high risk of acquiring HIV, for example, or developing a mental health condition, or being subjected to partner violence, can seek advice immediately and, if necessary, be referred for professional help.

Level 3 Linking students to health and psychosocial services: These services include the toll-free crisis hotline for psychosocial support, more extensive professional counselling, and clinical services in the areas of HIV and TB prevention and treatment, sexual and reproductive health (SRH), and non-communicable diseases (NCDs). Some services are offered by campus health clinics and HIGHER HEALTH's mobile clinics, while others are accessed by referral to community health services.

Across the three levels of intervention, HIGHER HEALTH maintains a focus on areas of health that are particularly significant to young people in their late teens and early 20s. These are set out in the box on page 18.

The HIGHER HEALTH Model



Our strategy and operational model *continued*

HIGHER HEALTH focus areas



Mental health

Awareness, combatting stigma, risk assessment, access to services.



Gender-based violence

Awareness, risk assessment, early intervention, support groups, access to services.



Sexual and reproductive health

Contraception, pregnancy testing, access to antenatal care and termination services, prevention and early detection of certain cancers.



HIV, tuberculosis (TB) and sexually transmitted infections (STIs)

Awareness, risk assessment, screening and testing, prevention methods, access to treatment and support systems.



COVID-19 and epidemic preparedness

Disease prevention and control, including capacity building at institutions.



LGBTQI+ health

Awareness, promotion of rights, support groups, responsive health systems.



Harmful use of alcohol and drugs

Awareness, risk assessment, combating stigma, linkage to specialised services.



Health of people with disabilities

Awareness, advocacy and promotion of rights, support groups.

Closing the gap between knowledge and healthy living

One of the enduring challenges of health promotion is converting knowledge about healthy living into action. Global experience suggests that the following are important success factors:

- Taking account of the social environment in which individuals exercise their health decisions and building support within that environment.
- Acknowledging that change for healthier living requires specific skills and building pathways for individuals to acquire these skills.
- Understanding and respecting the key role of motivation in the process of personal change and finding ways to inspire a desire for change.

HIGHER HEALTH has embraced these learnings through our use of peer educators and student volunteers, campus health activations, campus radio stations, support groups, and tools such as self-administered risk assessments and a crisis helpline which can be used anonymously.

In addition, we have begun to emphasise the link between civic responsibility, a healthy society, and the health of individuals. This approach emerged from our lived experience. Our HIV programmes sensitised us to the extent to which human rights abuses and discrimination increase vulnerability to infection and deter health-seeking behaviour. Our work in the area of GBV highlighted the power of public policy and social norms in shaping an effective response. COVID-19 compelled us all to become each other's keepers, to act responsibly in order to protect ourselves and our fellow citizens.

During the past year, we sharpened our focus on civic values and responsibilities. To start, a group of student leaders participated in a memorable experiential learning workshop on Robben Island during the week of the State of the Nation address. In future, a civics module will be offered to students more broadly as part of our accredited National Civic Education and Health Skills Programme.

Our partnership with universities, TVET colleges and CET colleges

Empowering PSET institutions to deliver health and psychosocial programmes takes place in various ways, including offering technical guidance, building capacity among their staff members and student leadership, and providing resources to facilitate programme implementation.

Technical guidance is provided through the development of strategic frameworks to address priority areas of health, guidelines to support implementation of these strategies, and – where necessary – detailed protocols and administrative tools. The creation of high-level strategies is a consultative process which HIGHER HEALTH enables by enlisting experts in relevant fields and facilitating their engagement with PSET institutions. Our most notable contributions at this level have been in building the sector's responses to GBV and COVID-19 in alignment with national policy.

Capacity building of campus management, staff and student leadership for the implementation of strategies and the HIGHER HEALTH Model is essential and we devote considerable energy to this. The intensity and detail of training vary in accordance with the roles various groups are expected to play in executing strategies, guidelines and protocols.

Providing **supplementary resources** to institutions varies across PSET sub-sectors.

In the university sub-sector, institutions generally have greater human resource capacity to provide health and psychosocial services and clinic facilities for the delivery of care. Therefore HIGHER HEALTH supports universities through a monetary grant linked specifically to performance in delivering our programmes.

In the TVET college sub-sector, HIGHER HEALTH employs and seconds at least one campus health and wellness coordinator (CHWC) to every college. The role of the CHWC is to coordinate health and wellness activities on college campuses, to build capacity among staff, peer educators and student volunteers, to maintain relationships with local stakeholders, and to collect data on programme performance.

In addition, HIGHER HEALTH deploys a fleet of mobile clinics to selected TVET colleges to provide services to students on a roster basis. Mobile clinics are staffed by a professional nurse and nursing assistant.

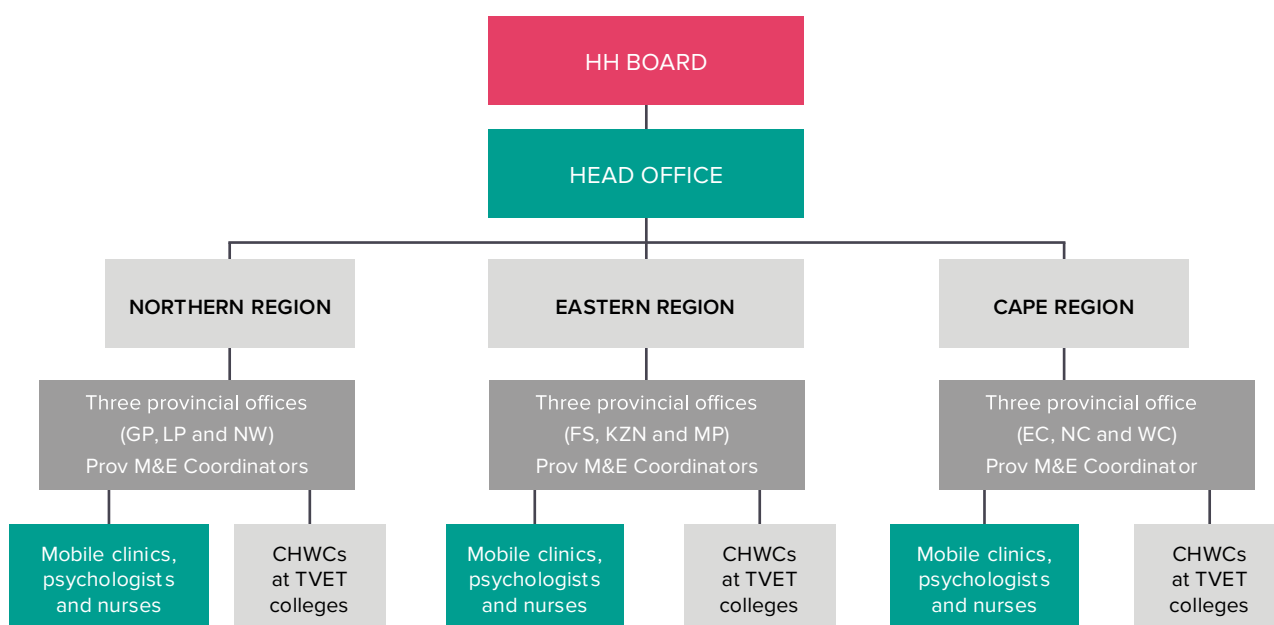
Support to CET colleges, with their diffuse network of educational centres, approximates the TVET model. The sub-sector has only been included in the HIGHER HEALTH programme for the past three years and the partnership is still evolving.

In all three sub-sectors HIGHER HEALTH supplies or mobilises the following resources to support implementation:

- Information, education and communication materials to support the health-awareness activities of peer educators and student volunteers.
- Training manuals and facilitator guides to support a train-the-trainer model of capacity development.
- Essential health products to support the clinical services – for example, condoms and lubricants, contraceptives, HIV testing kits, and – in some instances – medication for the treatment of HIV and TB. These items are supplied by provincial health departments through partnerships brokered by HIGHER HEALTH.

The structure that supports our work

HIGHER HEALTH's structure ensures that we have a presence in every province and that PSET institutions have direct face-to-face contact with our members of staff. Our provincial offices are responsible for the management of mobile clinics and CHWCs who service TVET and CET colleges.





SECTION 3:

DELIVERY OF CORE PROGRAMMES

DELIVERY OF CORE PROGRAMMES

Introduction

In contrast to the past two academic years, HIGHER HEALTH's attention in 2022 was no longer dominated by efforts to contain the COVID-19 pandemic. A high degree of COVID vigilance was necessary as the year got underway, but the extensive vaccination of students and staff encouraged a strong return to campus-based teaching and learning. Even the nationwide spike in COVID-19 during May and June 2022 proved manageable. Indications were that the country was living comfortably with this disease that had caused massive disruption only two years earlier.

The return to normality in 2022 was reflected in the resurgence of student participation in our other health and wellness programmes. While 2021 had seen a range of HIGHER HEALTH programmes recovering at TVET colleges under the guidance of community health and wellness coordinators (CHWCs), the university sub-sector took somewhat longer to regain lost ground. However, the revival of university activities was impressive in 2022.

In 2020, COVID-19 had been the catalyst for bringing CET colleges within the HIGHER HEALTH fold. Our commitment was to develop capacity at CET colleges for delivery of our full range of interventions and the data in this report indicate that we have gradually begun to do so. Although there are only nine CET colleges – one per province – the total number of learning centres attached to these colleges totals around 3 000 and fluctuates considerably from year to year. HIGHER HEALTH has focused on a limited number of larger CET learning centres and will gradually expand our reach. In 2022, we were active in 168 centres spread across all provinces.

Cross-cutting peer education activities

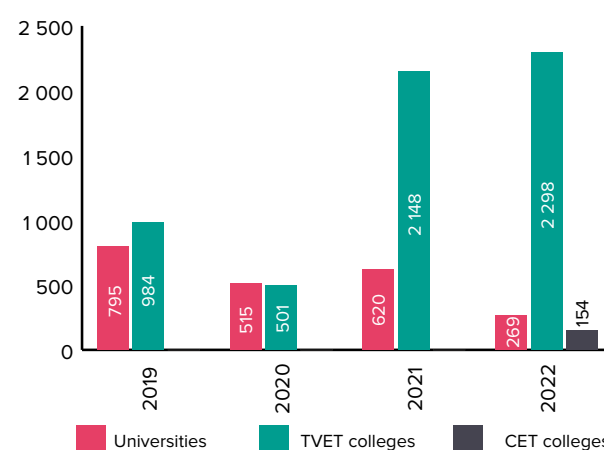
Peer-to-peer education is a major feature of almost all the programmes outlined below. The recruitment and training of students as peer educators is an ongoing process, with a new cohort entering the system every academic year. Student volunteers also fulfil other roles – for example, at the height of the COVID-19 pandemic many performed screening functions at entry points to campuses.

The benefits of peer education accrue not only to HIGHER HEALTH programmes – the method has proven effective within health promotion strategies – but also to students who volunteer, as they gain confidence and critical skills that will assist them in their personal, public and working lives.

At TVET and CET colleges peer educators and peer mobilisers work closely with CHWCs, who perform the more complex facilitation functions involved in offering the Co-Curriculum. At universities, peer educators are also entrusted with facilitating Co-Curriculum sessions.

In all instances, a major responsibility of peer educators is the organisation of health and wellness days on campus, under the banner of First Things First, during which educational activities and screening for various health conditions are offered in a highly accessible way.

Number of health and wellness days held on campus

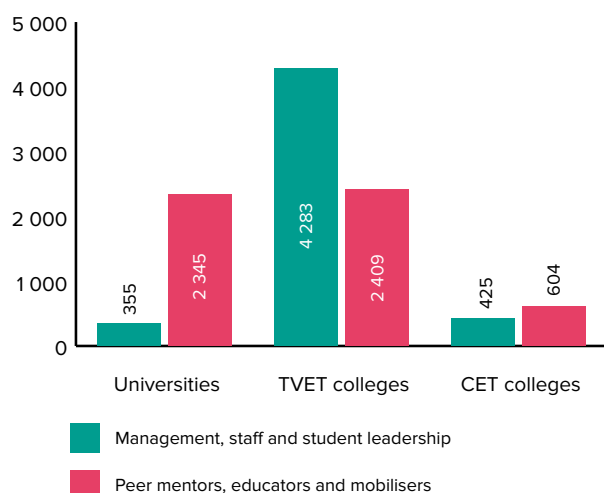


Building capacity to enable implementation

HIGHER HEALTH provides two broad types of capacity building. We educate university, TVET college and CET college management, senior staff and student leadership on the strategies, policies and guidelines we have developed in our major focus areas in order to secure their support for implementation. We also provide practical training for frontline staff, peer mentors and coordinators, and peer educators and mobilisers to enable them to put guidelines and protocols into practice, to conduct health days and campaigns, and help deliver the Co-Curriculum to students.

Delivery of core programmes *continued*

Attendance at training workshops for students and institutional staff 2022



HIGHER HEALTH ensures that its staff throughout the country are thoroughly trained in all areas of intervention and attendance at staff sessions totalled 1 019 in 2022.



Sexual and reproductive health

including HIV, TB and STIs

HIGHER HEALTH's approach to sexual and reproductive health (SRH) is to encourage and enable students to take responsibility and make informed choices within the context of respectful, safe relationships. We encourage young women to view SRH services as a tool for their empowerment and engage students of all genders and sexual orientations in a discussion on their rights regarding SRH.

Prevention is our primary objective in respect of HIV, other STIs and unplanned, early pregnancy because these often have lifelong consequences for young people, including the abandonment of their studies. Peer-centred information activities, Co-Curriculum sessions and self-administered risk assessments are the foundation of prevention. These go hand in hand with access to prevention services and products – condoms, other contraceptives, pre-exposure prophylaxis of HIV (PrEP), voluntary medical male circumcision (VMMC), HIV testing and appropriate counselling on the use of all these products.

A second major area is ensuring access to diagnostic and treatment services: STI screening, testing and treatment, antiretroviral treatment (ART) and other care for those who test positive for HIV, and pregnancy testing with counselling and referral either for antenatal care or termination of pregnancy, where necessary.

Included in this programme are awareness-raising, screening and referral for further investigation for breast, cervical, prostatic and penile cancer.

The final component is early TB diagnosis and treatment, included in this programme because of the strong link between HIV and TB. The emphasis has been on screening in order to identify the “missing” individuals who have TB, are unaware of it and therefore go untreated.

A critical success factor for the HIV and SRH programme is the “friendliness” of the clinic services for students. Many young people have grown up in households where sex is never discussed and this creates serious barriers to seeking care. If clinic staff are perceived to be judgmental of students' sexual behaviour or if they are insensitive to the specific needs of LGBTQI+ students, the willingness of students to seek care is severely undermined.

In the interests of easy access to care and quality control of services, HIGHER HEALTH would ideally like to see dedicated clinic services on every campus, provided either through our fleet of mobile clinics or facilities run by universities and colleges. At present this situation benefits a minority of campuses and we rely mainly on referrals to community health facilities – usually public clinics and hospitals, but also some private and NGO facilities.

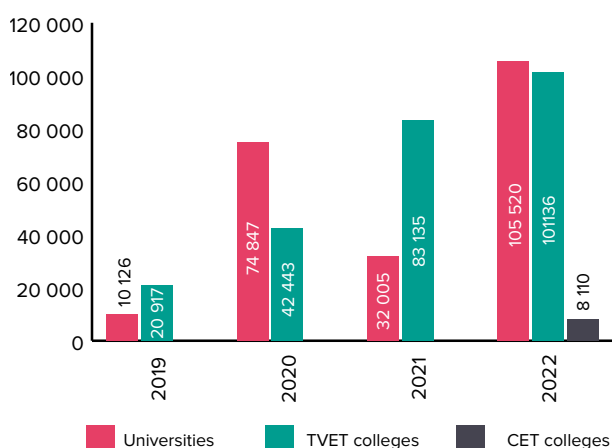
National and provincial health departments play a critical role in providing campus and mobile clinics with essential resources, such as contraceptives, condoms, HIV test kits and, in some instances, PrEP and medication for TB and ART. This in-kind donation by the public sector is a substantial contribution to the sustainability of PSET health services, particularly mobile clinics.

Student participation in Co-Curriculum sessions on HIV, TB and SRH increased substantially at universities and TVET colleges and got off to a promising start at CET colleges. At universities, the attendance more than trebled, largely due to the full-scale return to campus. This enabled universities to recruit and train many peer educators to deliver the Co-Curriculum.

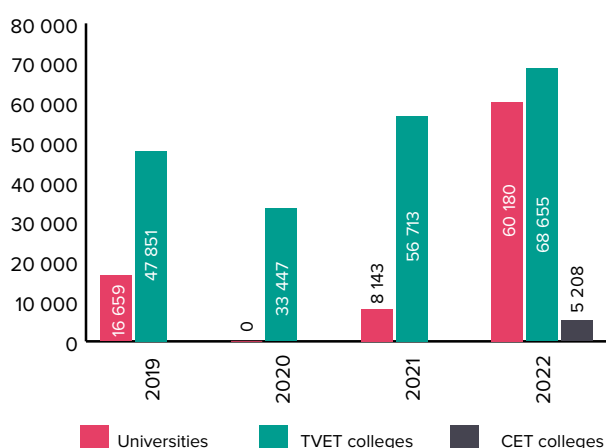
A similar pattern applied to student use of the risk-assessment tool for HIV, TB and SRH, with both TVET colleges and universities recording the highest participation levels to date. At universities, some flexibility in how the self-assessment tool was incorporated into the health and wellness programme appears to have encouraged uptake (see section on Universities Programme).



Participation in Co-Curriculum sessions on HIV, TB, STIs and SRH



Students completing self-administered risk screening for HIV, TB, STIs and SRH



The proportion of students considered to be at risk was considerably higher at universities than at colleges and the linkage to care was also more effective. This may be due to the availability of on-campus clinics and counselling services at universities and the integration of peer education activities into clinic services.

Risk screening and follow-up care: HIV, TB, STIs and SRH

Percent of screened students considered at risk

Universities

36%

TVET colleges

19%

CET colleges

6%

Percent of at-risk students linked to services and care

Universities

39%

TVET colleges

20%

CET colleges

18%

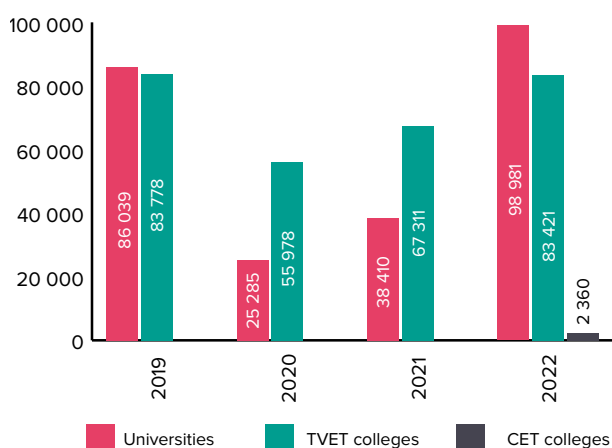
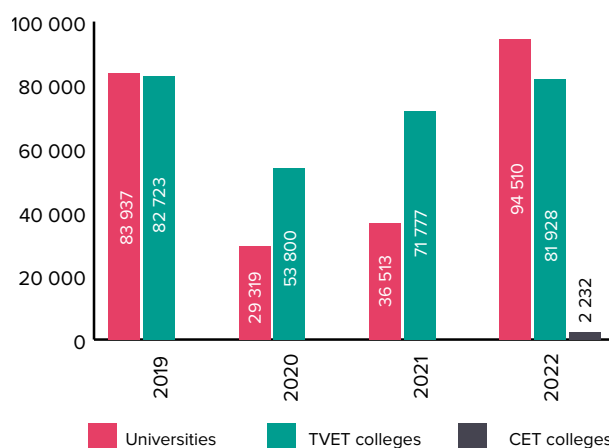
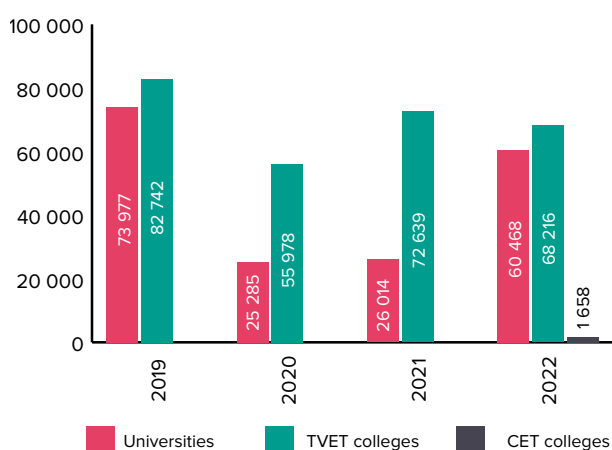
Delivery of core programmes *continued*

Countrywide, the COVID-19 pandemic disrupted HIV testing programmes in 2020 and 2021 and the PSET sector was no exception. In 2022 the sector saw a reasonable recovery in HIV testing – though pre-pandemic figures were not yet equalled. The HIV rapid test can only be administered by a health professional or trained HIV counsellor, which may have constrained access. STI and TB screening, on the other hand, can be performed by a peer educator and the uptake exceeded pre-pandemic figures at universities, while TVET colleges fell just short of the pre-COVID mark.

Since student enrolment at TVET colleges is less than half that at universities, the screening and testing at TVET colleges is reaching a higher percentage of students in the sub-sector. At CET colleges these interventions are relatively new and reach will expand as the programme matures.



Uptake of HIV testing and screening for TB and STIs



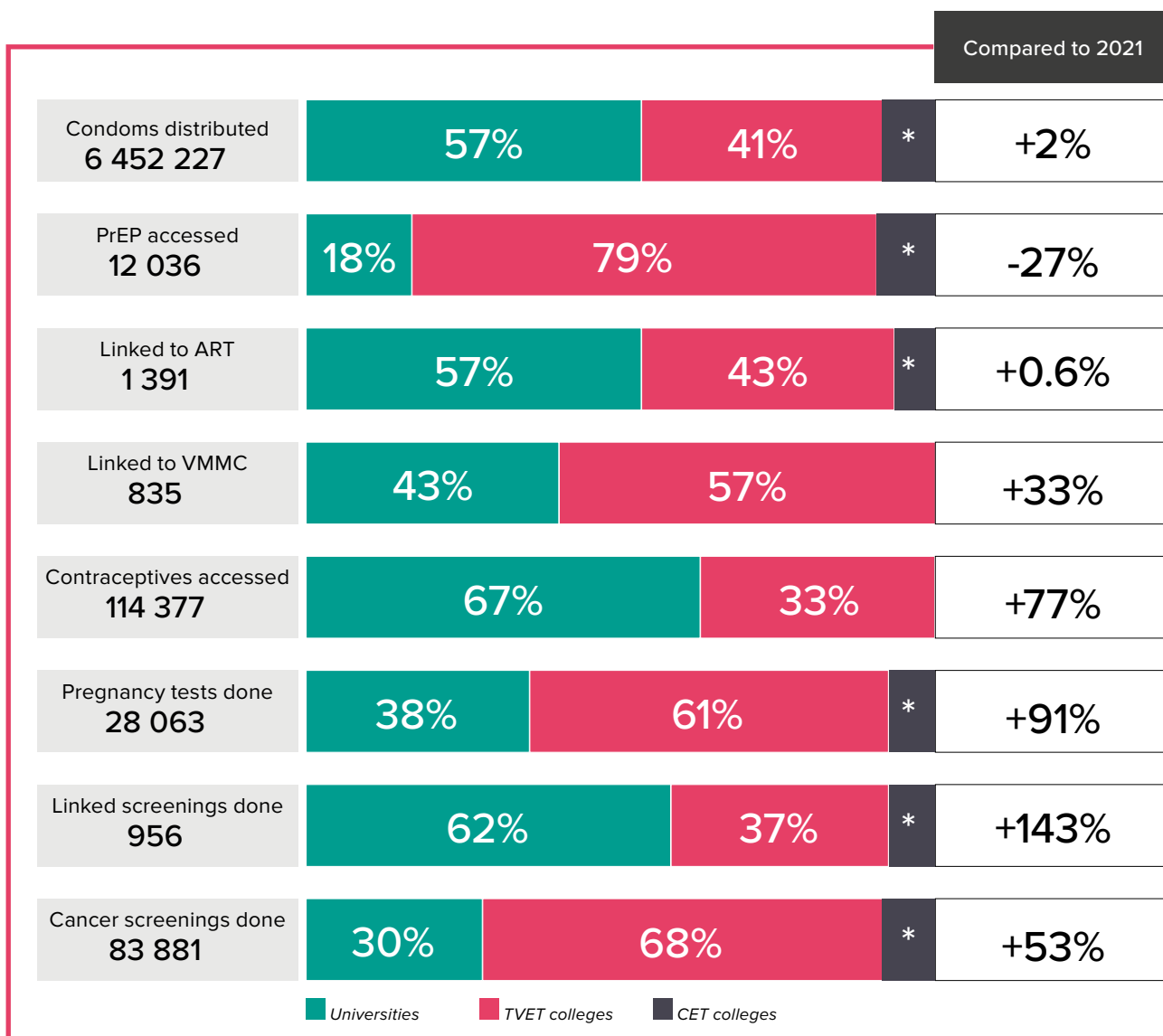
Universities and TVET colleges have sustained the distribution of a large quantity of condoms over the years. The overall figure in 2022 was 6.45 million and this included about half a million female condoms. Since condoms are the only product performing a triple function – preventing pregnancy, HIV and other STIs – they remain a mainstay of the HIGHER HEALTH programme.

It is concerning that the use of oral PrEP – the daily ARV pill to prevent HIV – was very low, especially at universities, and declined significantly from 16 409 in 2021 to 12 288 in 2022. Since PrEP does not require partner cooperation or consent, it has the potential to provide protection to young women who find it difficult to negotiate condom use with their sexual partners. This is a major advantage in a country where the rate of new infections is highest among young women.

The likely barriers to PrEP uptake include:

- Limited access: PrEP must be supplied by a health professional and on-campus clinic services are not

Access to SRH and HIV prevention products and services in 2022



* The share of products and services provided to CET college students ranged from <1% to 3%

universally available. Few campus clinics were able to provide PrEP and even some public health clinics were unable to offer PrEP during 2022.

- **Personal preferences:** It is likely that individuals may be hesitant to take a daily ARV pill without a compelling medical need and that they may find the daily regimen inconvenient, doubting their own ability to adhere to it. There are indications that quite a high proportion of students who start PrEP are unable to sustain usage.

In terms of contraception, condoms continue to play an important role and the uptake of additional methods is gaining ground, with the figure virtually doubling across the sector. Injectables are the most used form of contraception among students (apart from condoms),

followed by the multiyear subdermal implant. Uptake of the daily oral contraceptive pill is relatively low.

Global Fund Adolescent and Young People's Programme

HIGHER HEALTH is fortunate to have received a new three-year grant from the Global Fund to Prevent HIV, TB and malaria. In contrast to our previous Global Fund grant, which focused on HIV prevention among adolescent girls and young women, this allocation includes both male and female students attending TVET colleges in 14 sub-districts of the country. The grant is channelled through three major NGOs: the Networking AIDS Coalition of South Africa (Nacosa), the AIDS Foundation of South Africa (AFSA), and Beyond Zero.

Delivery of core programmes *continued*

The objectives of the grant are to:

- Reduce new HIV infections among young people.
- Reduce teenage pregnancy and other unwanted pregnancies.
- Reduce GBV.
- Increase the retention of students in PSET.
- Improve economic opportunities for young people.

With the broad goal of contributing to a healthy, educated, empowered and safe young generation, the Global Fund grant is strongly aligned with HIGHER HEALTH's mission.

The three key performance indicators for the grant indicate the Fund's expectations of HIGHER HEALTH. These indicators are: the number of students who take an HIV test and receive their results, the number accessing a package of HIV prevention services, and the number of newly diagnosed HIV-positive students who enrol for ART.

The grant was intended to start in April 2022 but the effective start date was September as contractual formalities had to be concluded, additional staff engaged, and extensive training provided to new staff. Consequently, performance for 2022 was running significantly below target but improved markedly in the first quarter of 2023 thanks to specific plans to compensate for lost time.





The non-communicable diseases pandemic

HIGHER HEALTH is conscious that non-communicable diseases (NCDs) have become the greatest cause of mortality globally, accounting for nearly 74% of all deaths in 2019. Even in Africa, where communicable diseases are still the primary cause of death, NCDs accounted for 37% of deaths in 2019 and the figure continues to rise (WHO World Health Statistics 2022).

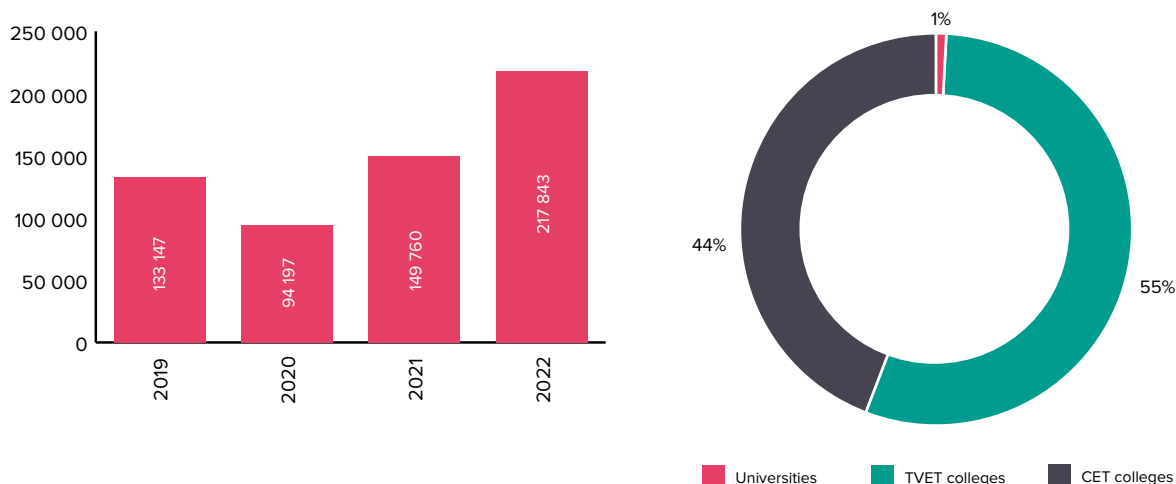
We are also aware that key risk factors for the major NCDs are lifestyle related and establishing healthy practices early in life may pay dividends as our student population ages.

While NCDs are mainly diseases that emerge later in life, diabetes is increasing among young people worldwide and in Africa there is a relatively high prevalence of elevated blood pressure among children and youth.

Although HIGHER HEALTH does not have a dedicated NCD programme, healthy living is part of peer-to-peer education and the Future Beats initiative, and screening for early detection of NCDs takes place during our First Things First health days and activations. There was a satisfying increase in the number of students screening for NCDs which speaks to an awareness of their significance. The PSET sector is playing a meaningful role in the national effort to prevent and control NCDs.

The figures below refer to screenings performed in relation to diabetes, hypertension, and cholesterol.

Number of NCD screenings performed



Delivery of core programmes *continued*

Gender-based violence

Gender-based violence (GBV) is a complex matter and a multisectoral approach is necessary to prevent and respond to violence of this nature. HIGHER HEALTH has played a key role in enabling such a response by helping to develop the PSET sector's policy framework on GBV and drafting a range of guidelines and protocols that help point all PSET role-players in the same direction.

During 2022, HIGHER HEALTH continued to facilitate a coordinated, comprehensive response by focusing on the training of university and college staff and student leaders to achieve consistent and correct implementation of the guidelines and protocols. These documents address many facets of the response to rape and sexual assault, sexual and gender-related misconduct, whistleblowing related to GBV and sexual misconduct, and the

implementation of minimum standards to prevent and respond to GBV on campus.

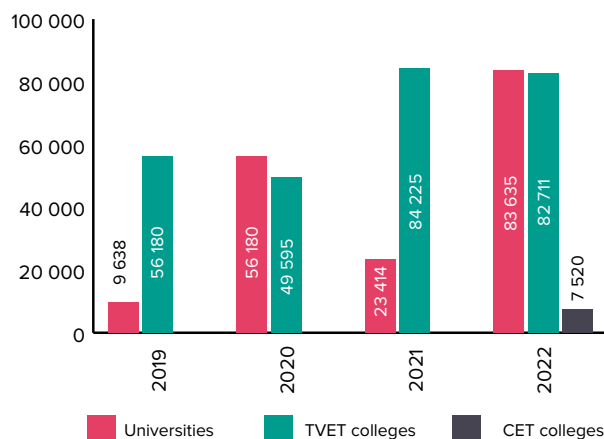
The 2 051 participants in the training workshops included campus health and psychosocial service staff, security personnel, human resources managers, residence managers, and student council members.

At the same time, the fundamental task of creating student awareness and understanding of GBV – and the related issues of gender equality and sexual harassment – continued apace with more students than ever before attending Co-Curriculum sessions and using the self-assessment tool to appreciate their personal risk of perpetrating or enduring GBV. Peer educators and the Future Beats radio programme continued to promote discussion on GBV.

Six pillars of the national policy framework on GBV in the PSET sector

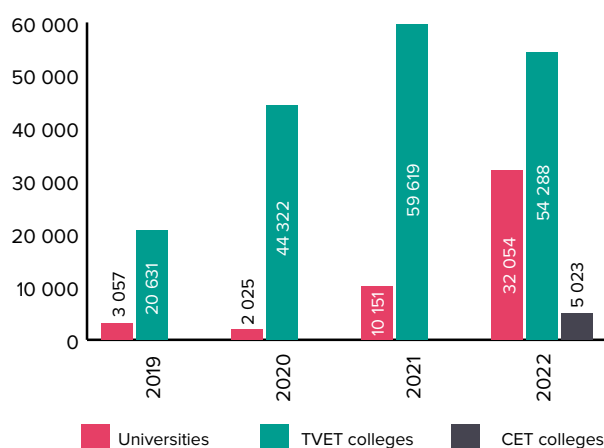


Number of students participating in Co-Curriculum on GBV



Note: Prior to 2022, GBV and mental health comprised a single module of the Co-Curriculum but in 2022 two separate modules were developed

Number of students completing self-assessment for GBV risk



Note: Prior to 2022, a single risk assessment tool was used for GBV and mental health but in 2022 two separate tools were developed

When self-screening indicates that students may be at risk of suffering or perpetrating GBV, they are referred for psychosocial support, either on campus or at an off-campus service. Referrals for psychosocial care are also made through the 24-hour crisis helpline and student health services.

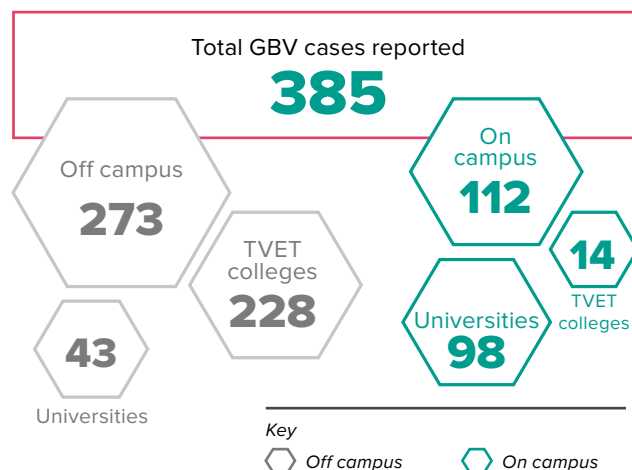
A total of 5 308 students were referred for psychosocial care related to GBV. The aim is to intervene early enough to prevent GBV or, in the case of students who have already experienced violence, to avoid continued abuse.

Prevention through early detection and psychosocial support

	Universities	TVET colleges	CET colleges
Students identified as at high risk of GBV through self-screening (2022)	733	3 154	73
Students referred for psychosocial support (2022)	1 407	3 741	160

A total of 385 instances of GBV were reported by students in the PSET sector in 2022, and these certainly do not represent the entire picture. About two-thirds of reported cases occurred off campus and mainly involved TVET college students. In contrast, GBV cases involving university students occurred mainly on campus. Universities have a high proportion of resident students and students spend more time socialising on campus. The challenges of protecting students off campus are infinitely greater because institutions have little influence on the physical and social environment beyond their gates.

GBV cases reported in PSET sector in 2022



Mental health and wellbeing

The first major national survey on the mental health of university students has affirmed HIGHER HEALTH's basic approach, indicating the need for a broad programme of mental health awareness-raising and for dedicated psychosocial services for a substantial number of students who experience a mental health disorder.

While most of the mental health stresses of the COVID-19 period have subsided, there are still academic pressures on some students whose studies were compromised by the long hiatus in contact teaching and learning.

Delivery of core programmes *continued*

Our mental health programme is structured on three levels of intervention:

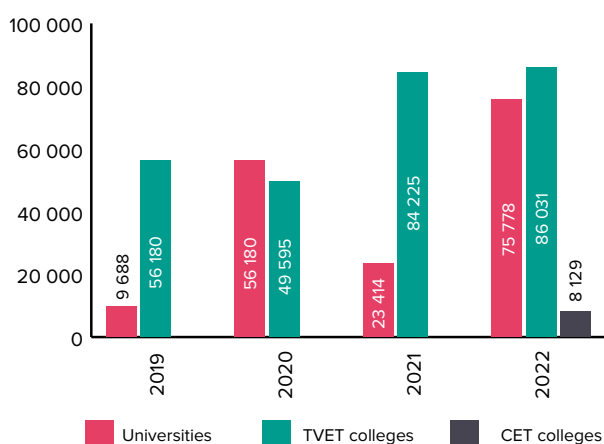
- Level 1 focuses on combating the stigma associated with mental disorders, creating awareness of mental health, providing psychoeducation, building resilience and self-help capacity, and modifying aspects of the campus environment that might affect mental wellbeing. We achieve these objectives through peer-to-peer dialogues, our Co-Curriculum on mental health, use of a mental health self-assessment tool by students, and developing guidelines and protocols to address risk factors on campus.
- Level 2 provides access to short-term crisis counselling and support. The vehicle for this is the toll-free 24-hour helpline which HIGHER HEALTH offers in partnership with the South African Depression and Anxiety Group (SADAG).
- Level 3 is concerned with offering professional care and making referrals where necessary for further care at specialised facilities. HIGHER HEALTH has clinical psychologists attached to our provincial offices. Universities and some bigger TVET colleges have psychologists and counsellors on their staff. Referrals are made to private practitioners (where students have medical aid) and state and private hospitals.

Response to Level 1 interventions

In 2022, participation in the mental health curriculum and the willingness of students to consider their personal mental health status were stronger than in previous years. This may be an indication that institutions are making headway in reducing the stigma attached to mental disorders and deepening understanding of psychosocial health.

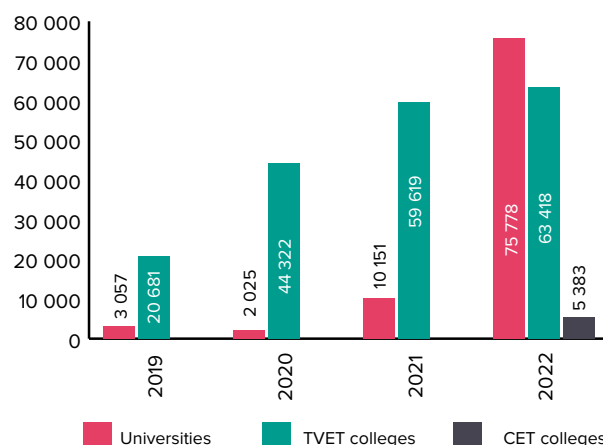
At TVET colleges and universities, about 8% of students who completed the self-assessment were considered at high risk of a mental disorder. The percentage at CET colleges was lower, at 2%.

Number of students participating in the Co-Curriculum on mental health



Note: Prior to 2022, GBV and mental health comprised a single module of the Co-Curriculum but in 2022 two separate modules were developed

Number of students performing self-assessment on mental health



Note: Prior to 2022, GBV and mental health comprised a single module of the Co-Curriculum but in 2022 two separate modules were developed

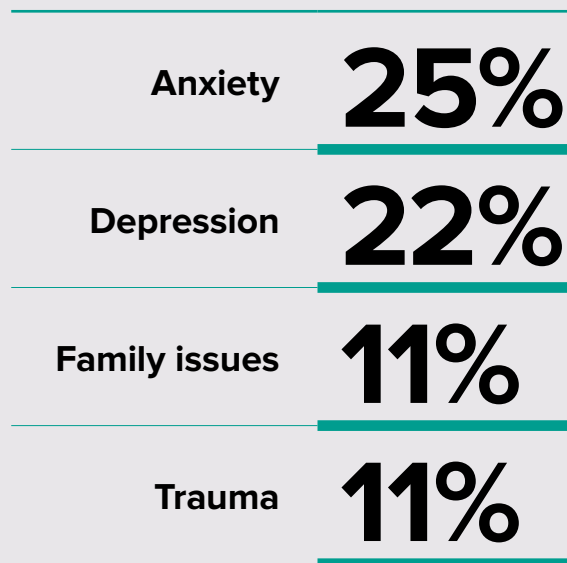
The value of the crisis helpline

The crisis helpline, established during the 2020 lockdown, is a multilingual service staffed by counsellors trained to respond to the immediate crisis that prompts a call and to link callers to services for further help where necessary.

In 2022, the helpline dealt with a total of **11 185 calls** and **670 SMS** appeals for help. This was similar to the numbers for 2021, which were **11 297** and **908** respectively.

Why students call the helpline

Four factors accounted for 69% of crisis helpline calls in 2022:



These figures resonate with data from the national student mental health survey.

Delivering psychosocial care

Psychosocial care is delivered directly by HIGHER HEALTH clinical psychologists based in some of our provincial offices and by psychologists and other professionals serving in campus clinics and student counselling units. In addition, students may be referred by helpline staff, CHWCs and peer educators to primary health care clinics and specialised mental health facilities in the community.

Nearly 15 000 students from across the sector were referred for help. HIGHER HEALTH's psychologists conducted a total of 11 185 sessions with students, translating to an average of 858 a month. Some were individual sessions and others were conducted with groups of students. HIGHER HEALTH offers a time-limited service, allowing a maximum of six sessions per student. Those requiring prolonged support are referred to external service providers.

The need for mental health support services far exceeds the number of psychologists currently available. HIGHER HEALTH has seen the resignation of several psychologists and experiences difficulty recruiting to fill these vacancies as our funding only covers entry-level professionals. We have therefore introduced task-shifting which involves contracting registered counsellors who are able to undertake some of the work of clinical psychologists.

Supporting vulnerable populations

Just as in the wider society, social and economic factors play a significant role in the health and wellbeing of students. There is a large body of research that confirms that the physical and psychosocial health of sub-populations and social groups that are marginalised or disempowered is generally poorer. HIGHER HEALTH has identified four sections of the student population as vulnerable:

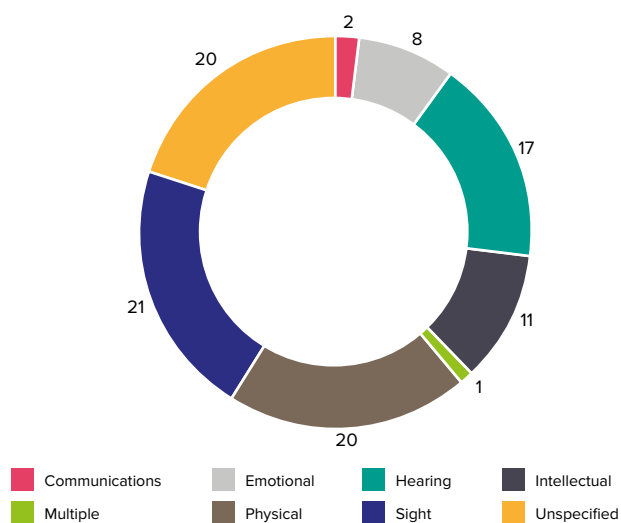
- Young women.
- The campus LGBTQI+ community.
- Students with disabilities.
- Students with harmful drug and alcohol habits.

Each of the above groups requires a unique approach, but in all cases HIGHER HEALTH's interventions are human rights-based and seek to combat stigma and promote inclusivity. The Global Fund Adolescent and Young People's Programme caters for young women students, and our approach to each of the other groups is outlined on the following pages.

Students living with disabilities

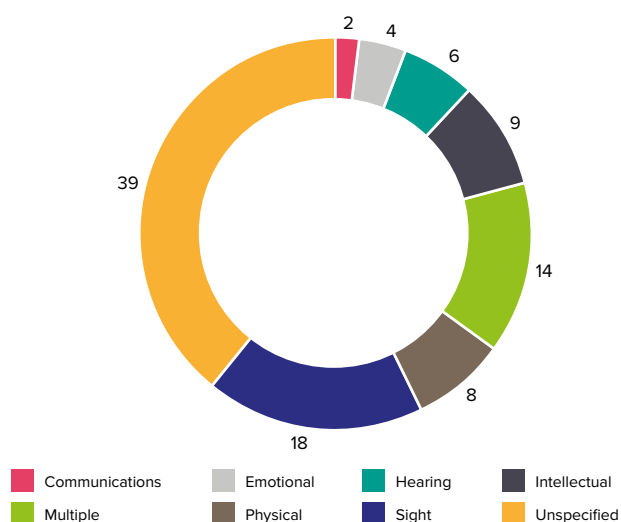
Nearly 16 000 students enrolled at universities and TVET colleges in 2020 reported having a disability. At universities 1.1% of the total number of students reported having a disability and at TVET colleges 0.94%. While the more apparent disabilities – such as visual, hearing and physical impairment – are relatively common, several less visible disabilities also affect students.

Primary disability reported by university students (%)



Source: Statistics on post-school education in South Africa 2020. DHET, 2022.

Primary disability reported by TVET college students (%)



Source: Statistics on post-school education in South Africa 2020. DHET, 2022.

Delivery of core programmes *continued*

Prevention of alcohol and drug abuse

Research indicates that about one in three young South Africans consumes alcohol before attaining the legal age of 18 years. Alcohol use among students entering PSET institutions is therefore quite pervasive.

In South Africa, the more serious concern is the high rate of heavy episodic drinking – commonly referred to as binge drinking – among men of all ages who consume alcohol. This carries increased risk of harm.

The recent national student mental health survey found that 2.5% of university students were alcohol dependent and 5.1% abused other substances.

Common wisdom is that younger, healthier bodies can stand a bit of hard living and abuse. But, when it comes to alcohol, this is not true – at least when it comes to the impact on the brain. Parts of the human brain do not fully mature until about the age of 25 years. This means impairment of judgment and coordination due to alcohol consumption is likely to be greater in young people and that repeated binge drinking can result in brain damage.

Immoderate alcohol use is associated with road accidents, risky sexual behaviour, mental and physical illness, and committing and falling victim to acts of violence (including GBV).

HIGHER HEALTH's programme aims to reduce alcohol misuse by raising awareness of the potential harm and to extend assistance to students who are alcohol dependent or at risk of dependence. The USAf/SAMRC survey found that only one in four students using alcohol and drugs in a harmful way was receiving treatment.

Support for LGBTIQ+ students

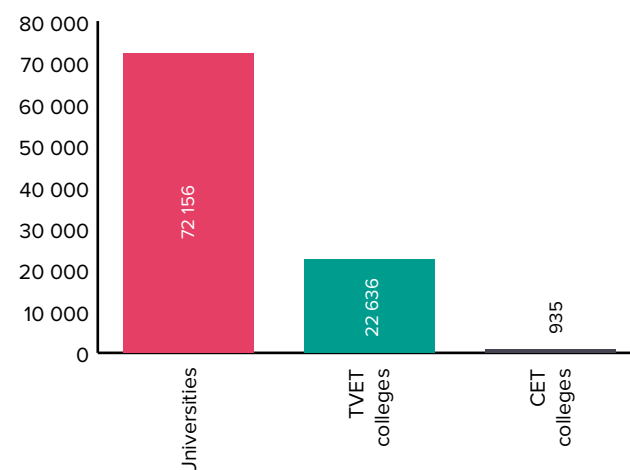
Discrimination against LGBTIQ+ students on campuses sometimes goes beyond exclusion to outright hostility. There is no definitive national research on the size and nature of the LGBTIQ+ component of our national student community, but the obstacles they face have been documented in small studies and by advocacy initiatives. These include labelling, stigmatisation, threats of sexual violence, and reduced access to resources, such as places at campus residences. Consequences in terms of the mental health of many LGBTIQ+ students have been observed – a pervasive sense of being unsafe, a feeling of alienation or not being at home on campus, diminished self-esteem and possible withdrawal.

HIGHER HEALTH's interventions in relation to all three vulnerable groups is to work to create a more hospitable and affirming environment on campuses across the country and to support and build resilience among members of these groups. Activities include the following:

- Sensitising and educating the broader PSET community about people with disabilities, LGBTIQ+ rights and needs, and the nature of substance misuse. This occurs through peer-to-peer engagement, Future Beats discussions, and a dedicated Co-Curriculum module.
- Organising support groups through our peer educator network so that people who have disabilities, are gender non-conforming or belong to a sexual minority have safe spaces where they can express themselves more fully and without fear.
- Supporting advocacy initiatives for physical changes to campuses and policies that would facilitate inclusion – for example, providing ramps and lifts, expanding access to learning materials in various formats, introducing gender-neutral toilets and residences.
- Equipping healthcare providers to respond to the specific health needs of young people with disabilities and LGBTIQ+ students. Lack of understanding of their needs often denies these students effective health services.

However, HIGHER HEALTH acknowledges that our vulnerable populations programme requires further development and upscaling. Participation in the Co-Curriculum module on alcohol use, disability, and gender and sexual diversity was only half that achieved for other modules.

Participation in Co-Curriculum sessions on alcohol, disability, and sexual and gender diversity 2022



Maintaining COVID-19 and epidemic preparedness

The mass uptake of vaccines that hugely reduced COVID-19 hospitalisations and deaths, together with progressively mild variants of the virus, made it possible to return to full-scale on-campus teaching and learning in 2022. But this did not mean that the PSET sector could let its guard down. COVID-19 was still with us – as the infection spike in May and June confirmed – and lecture halls, classrooms, canteens, and residences were environments where even the milder variants could prove extremely disruptive unless precautions were maintained.

HIGHER HEALTH continued to drive capacity building for infection control among institutional staff and student volunteers. This included keeping management and academic staff abreast of the evolving pandemic and resulting changes in regulations. The scale of this activity, relative to the previous year, is reflected in the table.

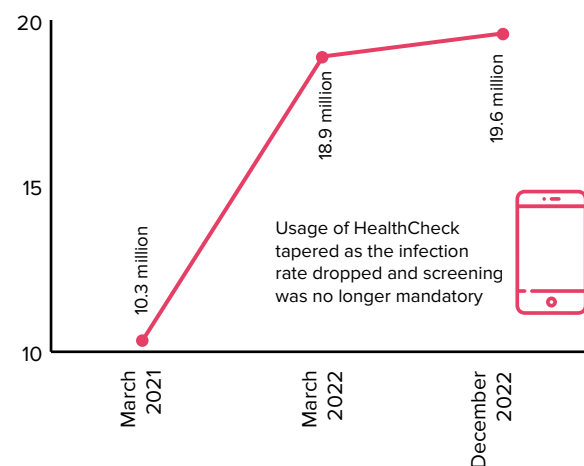
Training of staff and students for COVID-19 response

Category of personnel and volunteers	2022	2021
Institution management and academic staff	12 354	21 635
Screening volunteers and peer educators	4 569	21 336
Frontline staff	3 921	2 780
All categories	20 844	45 751

We encouraged institutions to continue to promote vaccination against COVID-19. Several universities had introduced vaccination mandates which required all students to provide proof of vaccination as a condition of registration for the 2022 academic year. Elsewhere, HIGHER HEALTH encouraged universities and colleges to promote vaccination positively.

There is no accurate measurement of COVID-19 vaccination coverage in the PSET sector. Students were invited to log their vaccination status on the digital COVID-19 access control tool, Health Check, and some 3.73 million reports were filed by the end of December 2022. However, it is not possible to tell how many students these reports represent. It is worthwhile noting that the proportion who were fully vaccinated (49%), partially vaccinated (17%) and unvaccinated (33%) did not shift between March and December 2022.

Utilisation of mobile COVID-19 app: cumulative HealthChecks performed



Monkey pox alert

One of the enduring lessons of COVID-19 has been the value in providing members of the public with good quality information that enables them to protect themselves and seek care when necessary. When the international outbreak of monkey pox occurred in 2022, HIGHER HEALTH was swift to utilise the mass media and organise webinars to provide information.

The University Programme

HIGHER HEALTH offers financial and technical support to the 26 public universities to implement a comprehensive health and psychosocial programme based on our model of health promotion. One of the outstanding achievements of 2022 was the massive scale-up of delivery as universities seized the opportunities presented by the complete resumption of face-to-face teaching and learning.

Grant-making to universities

In April 2021, HIGHER HEALTH earmarked a total of R10.96 million for disbursement to universities over a period of two years. Individual grants ranged from R250 000 to R500 000 per university and depended on the size of the institution, the business plan submitted, and the capacity of the institution to utilise the amount. An amount of R7.9 million, representing 72% of total allocation, was disbursed by the end of the funding cycle in March 2023.

The continued release of funds in tranches was dependent on universities meeting specified output targets, namely:

- The number of students participating in Co-Curriculum sessions. By the end of the grant cycle in March 2023, the average score of all participating universities against the targeted figure was 90%.

Delivery of core programmes *continued*

- The number of students conducting self-assessments of various health risks. Although risk screening improved dramatically compared to 2021, universities on average reached only 48% of the targeted figure by the end of the grant cycle.
- The number of at-risk students linked to treatment, care and psychosocial services. On this count, universities accomplished an average of 61% of the targeted figure by the end of the grant cycle.

The above figures represent the average for the sub-sector and some universities performed more strongly than others.

A fruitful approach adopted in 2022 was the blending of digital service delivery methods developed by universities during the pandemic with more traditional face-to-face health promotion interventions. HIGHER HEALTH recognised there was much to be gained by diversifying the platforms for delivering the Co-Curriculum and completing health risk assessments. This flexibility contributed significantly to the ability of universities to scale up these interventions.

HIGHER HEALTH decided to extend the grant cut-off date to 31 May 2023, which would allow institutions time to wrap up outstanding deliverables and be eligible to receive the final grant instalments.

Capacity development and technical guidance

HIGHER HEALTH provides training for the delivery of all its programmes to relevant staff members and student leaders, broadcasters, peer educators and volunteers. The categories of staff participating in these training workshops include executive and senior management, academic staff, student affairs personnel, campus health and wellness staff, wardens of residences, campus protection services and various frontline workers.

Building capacity among staff and students for the introduction of GBV guidelines and protocols was a particular focus in 2022, with universities showing a strong interest in coming to grips with this complex issue.

Sectoral coordination and stakeholder engagement

The pandemic took its toll on the functioning of institutional health and wellness committees, the key structures for planning and coordinating interventions at every university and ensuring the buy-in of senior management. HIGHER HEALTH applied itself to the reconstitution of established committees and the development of new committees. This is necessarily slow work, but good progress was made at several universities during the year and efforts will be sustained in the year ahead.

Consultations on plans for an accredited National Civic Education and Health Skills Programme were held with deans of students, deans of health sciences, campus health and wellness programme managers, project implementers and experts on different aspects of this proposed curriculum.

Meetings with departments of health at national, provincial and district level were held to explore ways to expand the provision of PrEP, ART and SRH services on campuses.

The TVET College Programme

HIGHER HEALTH provides human resources directly to TVET colleges by seconding CHWCs from our provincial offices to every college and staffing mobile clinics that service selected TVET colleges on a roster basis. We also build capacity among college staff and student peer educators and mobilisers to ensure that management and other staff members understand and support our interventions, and to develop implementation capacity on the ground.

CHWCs, working with peer mentors, educators and mobilisers, have been very effective in delivering levels 1 and 2 of the HIGHER HEALTH Model – that is, knowledge transfer and practical learning. Generally, colleges improved slightly on the high levels of student participation they had achieved in 2021. Given that their enrolment constitutes only one-third of the total number of students at institutions supported by HIGHER HEALTH, TVET colleges punch above their weight in terms of the student response to health and wellness activities.

However, when it comes to level 3 of our model – linking students to youth friendly clinical services – the constraints on offering on-campus care affect performance in this sub-sector. In 2022, HIGHER HEALTH's mobile fleet comprised just 10 clinics which meant we could only cover high-volume campuses with an enrolment exceeding 3 000 and could not visit as frequently as we would like. The COVID-19 pandemic reversed many of the gains we had made earlier in collaborating with district health services to run clinic sessions at colleges.

Consequently, we relied on referring students to community health facilities for clinical tests and treatment. These health facilities vary considerably in terms of standards of patient care. Some cope with heavy patient loads, which may affect service quality, not all follow youth friendly prescripts, and it is not simple to collect data on the services provided to those we refer. The Global Fund grant to our partner organisations, specifically to provide support to district health services, is expected to help address these issues.

Therefore, throughout the past year, HIGHER HEALTH continued to:

- Seek resources to sustain and possibly expand the mobile clinic fleet.
- Interact with provincial health authorities on entering into formal agreements on the extension of services to the TVET sub-sector.

There were some encouraging developments in the Western Cape where the West Coast College mobile clinic was launched as part of the outreach programme of the provincial health department and will provide services to all the campuses in the district.

Reinstating provincial forums

Prior to COVID-19 provincial forums for TVET colleges, their healthcare partners and HIGHER HEALTH staffers provided important opportunities for networking, sharing of experiences, and shaping of programmes. The pandemic effectively collapsed the forums and work on reconstructing them began in the past year, with North West and Mpumalanga taking the lead.

CET College Programme

HIGHER HEALTH has been awarded grants by the DHET and the HWSETA to establish our tried-and-tested health and wellness programmes in the CET sub-sector. The CET Programme was initiated at the height of the COVID-19 pandemic and initially focussed strongly on measures to prevent transmission of the virus among students and staff. By the start of 2022, however, we were able to offer a range of additional health and psychosocial services at 168 larger CET centres.

The CET sub-sector delivery systems are like those in the TVET sub-sector, in that CHWCs attached to HIGHER HEALTH's provincial offices are deployed to support a cluster of CET centres and capacity building is offered to staff and students to ensure they are fully involved in organising health days, conducting health promotion campaigns, and mobilising student participation in Co-Curriculum session and health risk screening.

- A group of 25 CHWCs were recruited, trained and assigned to work with CET centres.
- 604 peer educators and peer mobilisers from the relevant CET centres underwent training in the established HIGHER HEALTH method and programme areas.
- Extensive training was provided to management, various categories of staff, and student volunteers to ensure guidelines and protocols on COVID-19 were implemented, screening at centres was observed, vaccination was promoted, and individuals were linked to care where necessary.

- CET college students in some areas were able to access mobile clinics.

Communication Programme

The HIGHER HEALTH Communication Programme has two distinct components: a health promotion component which educates students and promotes discussion to enable them to make decisions on matters affecting their health; and an advocacy and stakeholder relations element designed to advance HIGHER HEALTH and build support for its approach to student health and wellbeing.

Communication for student health

The Future Beats initiative, which leverages the influence of campus and community radio stations to educate and encourage students to pursue good health, remains our key media-based method of health promotion. This is supported by use of social media and the production and distribution informative, educational materials.

HIGHER HEALTH partners with selected campus radio stations and community stations serving areas with large student populations. Participating stations vary slightly from year to year, but we have formed enduring working relations with a core group.

We make a modest grant to each participating station and provide training to their producers and presenters on our priority health subjects. In return, the stations produce regular programmes on health and wellness. They follow a content calendar developed by HIGHER HEALTH, which helps to ensure that the topics discussed on air cover all the bases we would like included. Crucially, however, the student journalists bring their unique creativity and insights on their audiences to the project. Each Future Beats station has its distinctive way of addressing health issues.

The combined listenership of the 20 stations participating in 2022 was 785 841 and their social media reached a further 239 367. In some instances, the number of people following the station on social media exceeded their listeners. While the size of the audience is quite modest compared commercial radio stations, the value of Future Beats is that audiences are likely to be exposed to health content multiple times. This reinforcement of information and messaging is especially important when the objective is not merely to inform people, but also to shape attitudes and health-seeking behaviour.

Delivery of core programmes *continued*

Stations participating in Future Beats in 2022

Station	Type of station	Province	Number of listeners	Social media reach	Grant amount
Unitra FM	Community	EC	240 000	341	R30 000
Madibaz FM	Campus	EC	15 000	15 752	R20 000
Forte FM	Campus	EC	7 000	7 107	R20 000
VOW FM	Campus	GP	20 000	39 279	R60 000
Rhodes Music Radio	Campus	EC	21 838	21 838	R20 000
Alfred Nzo	Community	EC	138 000	6 796	R30 000
CUT FM	Campus	FS	38 000	49 114	R25 000
Kovsie FM	Campus	FS	28 003	9 683	R25 000
Barberton Radio	Community	MP	55 000	12 383	R30 000
SMU FM	Campus	GP	5 000	6 500	R20 000
Valley FM	Community	WC	121 000	15 000	R30 000
Univen Radio	Campus	LP	9 000	15 570	R20 000
Radio Turf	Campus	LP	6 000	14 000	R20 000
UCT Radio	Campus	WC	1 000	16 899	R20 000
Mdantsane FM	Community	EC	61 000	3 900	R30 000
Mahikeng FM	Community	NW	88 000	33 232	R30 000
Maputaland Radio	Community	KZN	85 000	6 913	R40 000
TUT FM	Campus	GP	54 000	7 500	R30 000
Tshwane FM	Campus	GP	38 000	27 512	R30 000
Emalahleni FM	Community	MP	97 000	20 796	R30 000
Total			785 841	239 367	R560 000

Each year HIGHER HEALTH produces posters and concise leaflets to raise awareness and inform students about various health conditions. We also supply branding items to publicise health days and T-shirts to identify peer educators and mobilisers.

Media advocacy and stakeholder engagement

HIGHER HEALTH has been adept at gauging moments when media interest can be focused on issues related to the health of young people – and students in particular – and has proactively engaged journalists at these junctures. As a result, the traditional mass media have been an important tool for raising public awareness of the vulnerabilities of students and the need to develop programmes to protect their wellbeing.

The emergency of monkey pox in countries where this infection had been absent elicited several queries from the media and Prof Ahluwalia played a role in providing information and allaying fears.

In 2022, HIGHER HEALTH added significant international activities to its strong programme of **engagement with partners and other stakeholders**. We continued to utilise digital platforms for some of this interaction but also took full advantage of the post-COVID freedom to travel the country and visit internationally.

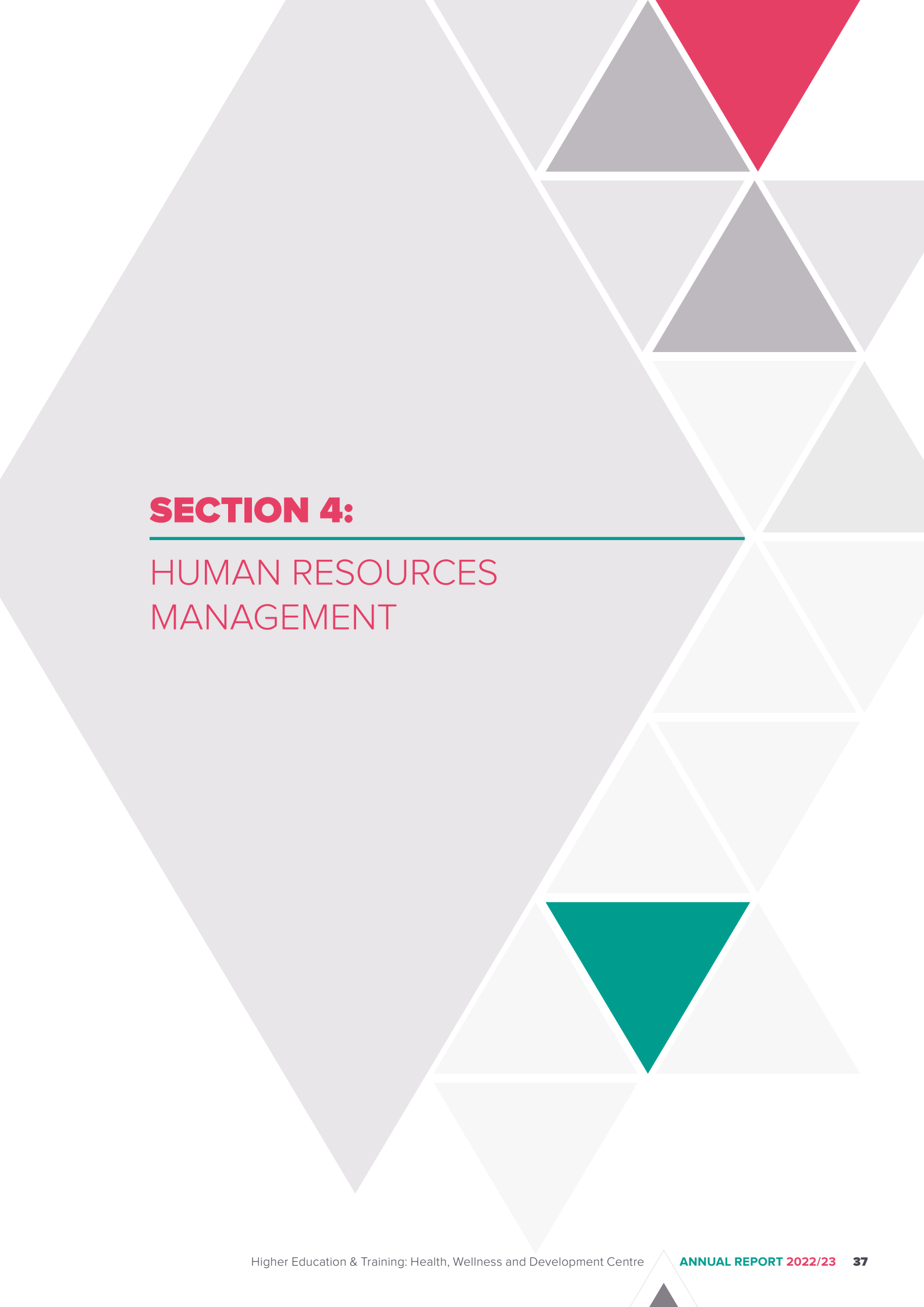
HIGHER HEALTH's excellent relationship with the DHET enables us to host Minister Nzimande and Deputy Minister Manamela from time to time at our national events. In early 2023, the Western Cape Provincial Minister of Health Nomafrrench Mbombo attended

a series of events at TVET colleges in the province, including the launch of a mobile clinic for the West Coast District.

While our priority is to host significant role-players in the health and PSET sectors at HIGHER HEALTH events on campuses where guests can see firsthand how institutions have embraced our model, we also appreciate invitations to present to various gatherings our experiences in building a national health and wellness programme in the PSET sector. Prof Ahluwalia takes every possible opportunity to network and advocate for responding to student health wellness needs in a developmental way that builds active citizenry.

His calendar for the year ranged from practical working meetings with groups such as the South African Senior Student Affairs Professionals (SAASSAP) and the Campus Protection Society of Southern Africa (CAMPROSA), to participation in high-level delegations led by Minister Nzimande and Deputy Minister Manamela. The latter included:

- Accompanying the Deputy Minister to Canada in September 2022 to attend the 10th Pan-Commonwealth Forum on Open Learning and forge links with York University.
- Representing South Africa at the WordSkills 2022 Special Edition in Lyon, France where youngsters with artisanal skills from all over the world competed.
- Joining Minister Nzimande's delegation to the 67th Session of the UN Commission on the Status of Women in March 2023.



SECTION 4:

HUMAN RESOURCES MANAGEMENT

HUMAN RESOURCES MANAGEMENT

“Busy” was an understatement for our human resources (HR) team as we strove to adjust policies and create systems to match our requirements as an organisation rapidly approaching 200 employees. The sheer pace of HIGHER HEALTH’s growth, the fact that much of this occurred during the exceptional COVID-19 period, and the uncertainties created by short-term grant funding had all contributed to a misalignment between HR management needs and practices.

Among the initiatives taken during the year to reduce the gap were:

- Moving from a situation where nearly all employees are hired on one or two-year contracts that match the length of grant funding, to one where a good proportion of employees have three to five-year contracts. We consider we have sufficient continuous funding to be able to achieve this without creating commitments that we would struggle to meet. However, the challenge will be developing rational and fair criteria for the award of these longer contracts.
- Creating a contract tracking process which enables us to manage the ending of contracts and the transition to new funding agreements more smoothly, with less of an impact on the job security of our employees.
- Taking the first steps towards acquiring a payroll system with greater functionality which will enhance the efficiency of the HR office.
- Developing and implementing new policy – for example on whistleblowing and compliance with the Protection of Personal Information Act (POPIA) – and reviewing existing policies, as required by our governance and internal audit provisions.
- Establishing an Employment Equity Committee.

Overview of the HIGHER HEALTH team

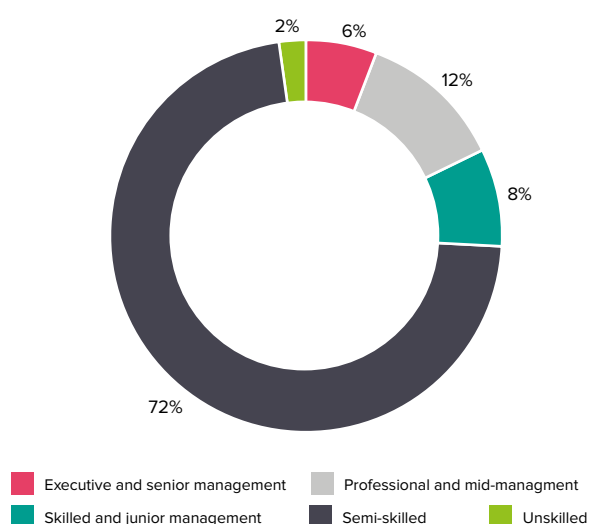
At the end of the 2022/23 financial year, HIGHER HEALTH had a total of 178 employees compared to 124 a year earlier. This growth of 43% was due mainly to expansion in the number of CHWCs for the CET sub-sector.

Our employees are broadly representative of the general population, with members of previously disadvantaged groups constituting the great majority of staff members. In terms of gender, 75% of HIGHER HEALTH’s team consists of women who form a majority at all levels of the organisation.

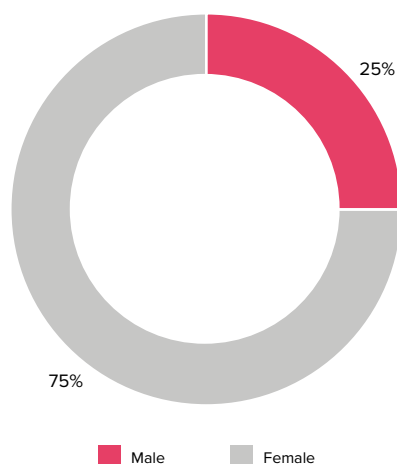
We have experienced difficulties in terms of retention of staff in some categories, notably clinical psychologists who form part of our service team for TVET and CET colleges. In some provinces, the demand for psychologists exceeds supply and those on our staff tend to leave for permanent positions with better

remuneration. We are looking to add social workers to our clinical teams in the future as they have the critical counselling skills we require and the supply-demand balance is more in our favour.

HIGHER HEALTH employees by occupational category as at 31 March 2023



HIGHER HEALTH employees by gender as at 31 March 2023





SECTION 5:

GOVERNANCE

GOVERNANCE

HIGHER HEALTH is a non-profit company, registered in terms of the Companies Act. The strategic direction of the organisation and oversight of its management and performance vest in a Board of Directors.

The Board has been constituted to include representation of stakeholders that have been key to HIGHER HEALTH's development (Universities South Africa [USAf], South African Public Colleges Organisation [SAPCO] and the Department of Higher Education and Training [DHET]) and to ensure that the organisation has access to individuals with specific expertise. The current chairperson is Prof Wim de Villiers, Vice-Chancellor of Stellenbosch University.

A full list of members appears below.

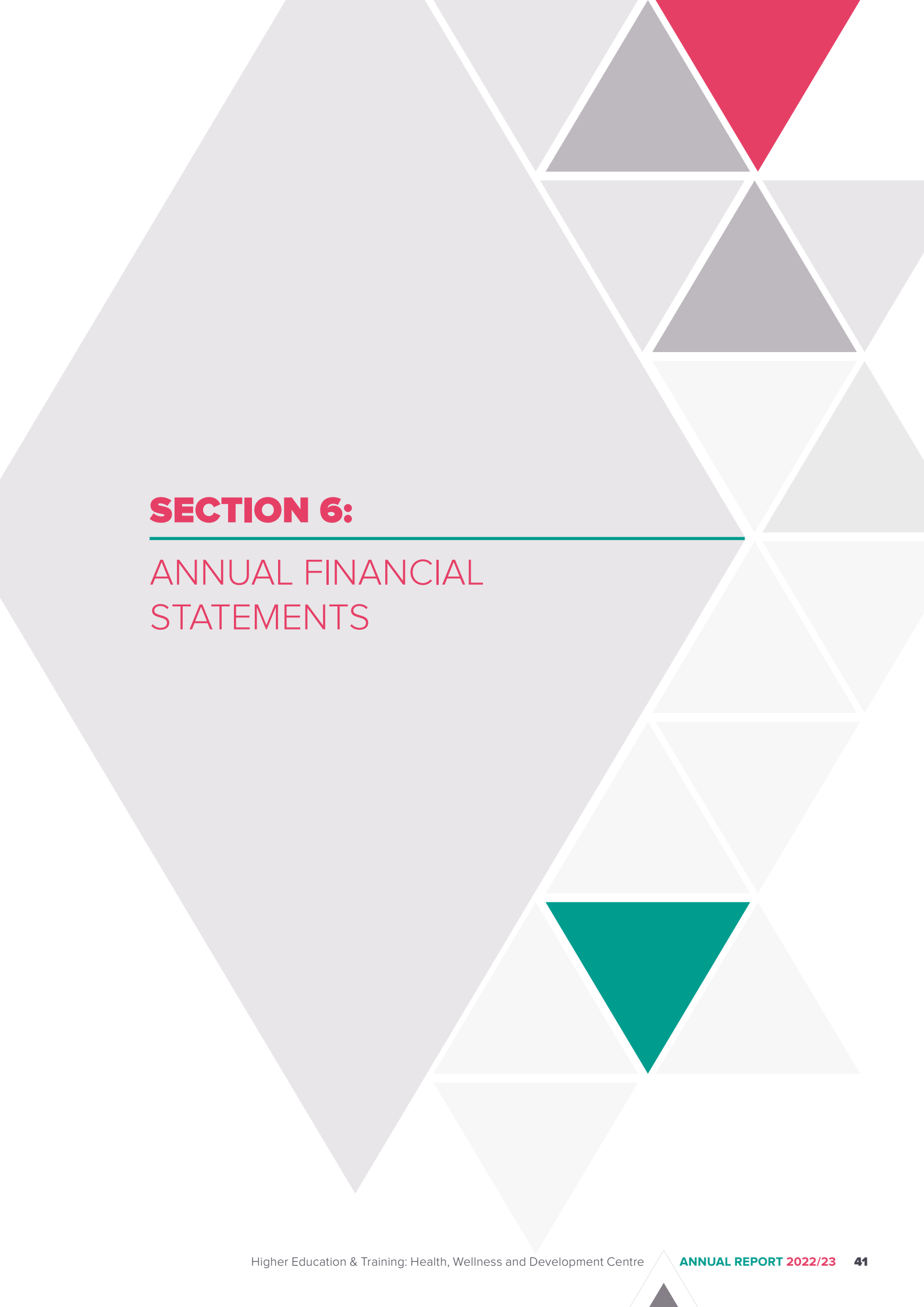
In terms of HIGHER HEALTH's memorandum of incorporation, the Board is required to meet at least three times a year and during 2022/23 it met three times.

Members of HIGHER HEALTH Board of Directors

Board member	Designation	Organisation or sub-sector represented/area of expertise
Prof Willem de Villiers Chairperson	Vice-Chancellor, Stellenbosch University	Universities/USAf
Mrs Maranna Dipiloane Monyadiwa Phutsisi Deputy Chairperson from 31 March 2023	Honorary Causa Principal, Motheo TVET College	TVET colleges/SAPCO
Dr Precious Petiwe Matutu	Chief Executive Officer, USAf	Universities/USAf
Ms Sesi Patricia Mahlobogoane	Director of Social Inclusion and Equity, DHET	PSET sector
Mr Xolelo B Phike	Principal, Northlink TVET College	TVET colleges/SAPCO
Prof Thokozile Valerie Mayekiso	Vice-Chancellor, University of Mpumalanga	Universities/USAf
Prof Nancy Refilwe Phaswana-Mafuya	Professor of Epidemiology and Public Health, University of Johannesburg	Public health
Ms Sibongile Mncwabe	Head of Corporate Services, National Student Financial Aid Scheme	PSET sector
Dr Doeke Tromp	Former CFO, Tshwane University of Technology	Financial management and strategic planning
Dr (Prof) Ramneek Ahluwalia <i>Ex officio</i>	CEO, HIGHER HEALTH	

The Board has established two committees: the Audit, Risk, Finance and Investment Committee, and the Human Resources and Remuneration Committee.

- Prof Mayekiso and Dr Tromp are members of the Audit, Risk, Finance and Investment Committee, which has an independent chair, Mr Theo Madurai, CFO of Tshwane University of Technology.
- Dr Matutu and Mrs Phutsisi are members of the Human Resources and Remuneration Committee.
- Prof Ahluwalia participates in meetings of both committees.



SECTION 6:

ANNUAL FINANCIAL STATEMENTS

GENERAL INFORMATION

Country of incorporation and domicile	South Africa
Nature of business and principal activities	Health and wellness services and capacity development in higher education institutions which include universities and TVET colleges
Directors	Dr D Tromp Prof W De Villiers (Chairperson) Dr (Prof) R Ahluwalia (CEO) Prof N Phaswana-Mafuya Prof T Mayekiso Ms Sesi Mahlobogoane Dr P Matutu Ms Sibongile Mncwabe Prof D Phutsisi Mr XB Phike
Registered office	262 Rose Avenue Wild Olive Riverfalls Office Park Centurion 0132
Business address	262 Rose Avenue Wild Olive Riverfalls Office Park Centurion 0132
Postal address	262 Rose Avenue Wild Olive Riverfalls Office Park Centurion 0132
Bankers	ABSA Bank Limited Investec Bank Limited
Auditors	PricewaterhouseCoopers Inc Chartered Accountants (SA) Registered Auditors
Company registration number	2017/419731/08
Tax reference number	9864/591/16/0
Level of assurance	These annual financial statements have been audited in compliance with the applicable requirements of the Companies Act of South Africa
Preparer	The annual financial statements were independently compiled by AfriZen Accounting and Consulting (Pty) Ltd

CONTENTS

The reports and statements set out below comprise the annual financial statements presented to the shareholders.

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AUDIT COMMITTEE REPORT

We are pleased to present our report for the financial year ended 31 March 2023.

The Audit, Risk, Finance and Investment Committee is an independent statutory committee appointed by the directors of the company. The committee has conducted its affairs in compliance with its terms of reference.

1. Members of the Audit Committee

The Audit, Risk, Finance and Investment Committee is independent and consists of five independent, non-executive directors. It meets at least twice per year as per its terms of reference.

The chairman of the Board, chief executive officer, finance director, external auditor and other assurance providers (legal, compliance, risk, health and safety) attend meetings by invitation only.

The members of the audit committee are:

Name	Appointed	Institution
Mr Theo Madurai	21/11/2017	Tshwane University of Technology – Chief Financial (Chairperson) Officer
Prof Ramneek Ahluwalia	21/11/2017	HIGHER HEALTH – Chief Executive Officer
Prof TV Mayekiso	21/11/2017	University of Mpumalanga – Vice-Chancellor
Dr Doeke Tromp	21/11/2017	Independent
Mr Eminos Manyawi	15/3/2019	Sefako Makgatho Health Sciences University – Chief Financial Officer
Mr Brian Madalane	15/3/2019	Northern Cape TVET College – Principal (Resigned prior to March 2023)
Mrs TJ Kula	2022	SAPCO Deputy President (Resigned March 2023) financial year

2. Meetings held by the Audit Committee

The Audit, Risk, Finance and Investment Committee performs the duties laid upon it by Section 94(7) of the Companies Act of South Africa by holding meetings with the key role players on a regular basis and by the unrestricted access granted to the external auditors.

3. External auditor

The Audit Committee has satisfied itself that the external auditor was independent of the company, as set out in Section 94(8) of the Companies Act 71 of 2008 South Africa, which includes consideration of previous appointments of the auditor, the extent of other work undertaken by the auditor for the company, and compliance with criteria relating to independence or conflicts of interest as prescribed by the Independent Regulatory Board for Auditors. Requisite assurance was sought and provided by the auditor that internal governance processes within the audit firm support and demonstrate its claim to independence.

The committee ensured that the appointment of the auditor complied with the Companies Act 71 of 2008 South Africa, and any other legislation relating to the appointment of auditors.

The Audit Committee in consultation with executive management, agreed to the engagement letter, terms, audit plan and budgeted audit fees for the 2023 year.

4. Annual financial statements

The audit committee has reviewed the accounting policies and the financial statements of the company and is satisfied that they are appropriate and comply with International Financial Reporting Standards.

5. Effectiveness and internal control

The system of internal control applied by the entity over financial reporting and risk management is effective, efficient and transparent. From the audit report on the financial statements and the management report of the external auditor, it was noted that no matters were reported that indicate any material deficiencies in the system of internal control or any deviations therefrom. Accordingly, the committee can report that the system of internal control over financial reporting for the period under review was efficient and effective.

On behalf of the audit committee



Theo Madurai
Chairperson of the Audit Risk Finance and Investment Committee

Monday, 2 October 2023

AUDIT, RISK, FINANCE AND INVESTMENT COMMITTEE

Directors' responsibilities and approval

The directors are required by the Companies Act 71 of 2008 South Africa to maintain adequate accounting records and are responsible for the content and integrity of the annual financial statements and related financial information included in this report. It is their responsibility to ensure that the annual financial statements satisfy financial reporting standards with regards to form and content and present fairly the statement of financial position, results of operations and business of the company, and explain the transactions and financial position of the business of the company at the end of the financial year.

The annual financial statements are based upon appropriate accounting policies, consistently applied throughout the company and supported by reasonable and prudent judgements and estimates.

The directors acknowledge that they are ultimately responsible for the system of internal financial control established by the company and place considerable importance on maintaining a strong control environment. To enable the directors to meet these responsibilities, the Board of Directors sets standards for internal control aimed at reducing the risk of error or loss in a cost-effective manner. The standards include the proper delegation of responsibilities within a clearly defined framework, effective accounting procedures and adequate segregation of duties to ensure an acceptable level of risk. These controls are monitored throughout the company and all employees are required to maintain the highest ethical standards in ensuring the company's business is conducted in a manner that, in all reasonable circumstances, is above reproach.

The focus of risk management in the company is on identifying, assessing, managing and monitoring all known forms of risk across the company. While operating risk cannot be fully eliminated, the company endeavours to minimise it by ensuring that appropriate infrastructure, controls, systems and ethical behaviour are applied and managed within predetermined procedures and constraints.

The directors are of the opinion, based on the information and explanations given by management, that the system of internal control provides reasonable assurance that the financial records may be relied on for the preparation of the annual financial statements. However, any system of internal financial control can provide only reasonable, and not absolute, assurance against material misstatement or loss. The going-concern basis has been adopted in preparing the financial statements. Based on forecasts and available cash resources the directors have no reason to believe that the company will not be a going concern in the foreseeable future. The financial statements support the viability of the company.

The annual financial statements set out on pages 11 to 27, which have been prepared on the going concern basis, were approved by the board of directors on 2 October 2023 and were signed on their behalf by:



Prof W De Villiers
Chairperson



Dr (Prof) R Ahluwalia
CEO

DIRECTORS' REPORT

The directors have pleasure in submitting their report on the annual financial statements of Higher Education Training Health Wellness Development Centre NPC for the year ended 31 March 2023.

1. Incorporation

The company was incorporated on 18 September 2017 and obtained its certificate to commence business on the same day.

2. Nature of business

Higher Education Training Health Wellness Development Centre NPC was incorporated in South Africa with interests in health and wellness services and capacity development in higher education institutions which include universities and TVET colleges. There were no major changes herein during the year.

The operating results and statement of financial position of the company are fully set out in the attached financial statements and do not in our opinion require any further comment.

3. Directorate

The directors in office at the date of this report are as follows:

Directors	Designation	Nationality
Dr D Tromp	Non-executive	South African
Prof W De Villiers (Chairperson)	Non-executive	South African
Dr (Prof) R Ahluwalia (CEO)	Executive	South African
Prof N Phaswana-Mafuya	Non-executive	South African
Prof T Mayekiso	Non-executive	South African
Ms Sesi Mahlobogoane	Non-executive	South African
Dr P Matutu	Non-executive	South African
Ms Sibongile Mncwabe	Non-executive	South African
Prof D Phutsisi	Non-executive	South African
Mr XB Phike	Non-executive	South African

4. Directors' interests in contracts

During the financial year, no contracts were entered into in which directors or officers of the company had an interest and which significantly affected the business of the company.

5. Property, plant and equipment

There was no change in the nature of the property, plant and equipment of the company or in the policy regarding their use.

6. Events after reporting date

All events subsequent to the date of the annual financial statements and for which the applicable financial reporting framework requires adjustment or disclosure have been adjusted or disclosed.

The directors are not aware of any material event which occurred after the reporting date and up to the date of this report.

7. Going concern

The annual financial statements have been prepared on the basis of accounting policies applicable to a going concern. This basis presumes that funds will be available to finance future operations and that the realisation of assets and settlement of liabilities, contingent obligations and commitments will occur in the ordinary course of business.

The directors believe that the company has adequate financial resources to continue in operation for the foreseeable future and accordingly the annual financial statements have been prepared on a going concern basis.

The ability of the company to continue as a going concern is dependent on a number of factors. The most significant of these is that the directors continue to procure funding for the ongoing operations of the company where the need arises. Management is comfortable that HIGHER HEALTH will continue to operate.

Further, Higher Health is actively involved in fighting against the pandemic and has been granted substantial funding to continue to do so. HIGHER HEALTH is in the forefront of rolling out a vaccine strategy especially in the PSET sector.

8. Independent Auditors

PricewaterhouseCoopers Inc continued in office as auditors for the year under review.

9. Secretary

No secretary has been formally appointed during the current financial year.

INDEPENDENT AUDITOR'S REPORT

To the Directors of Higher Education Training Health Wellness Development Centre NPC

Our opinion

In our opinion, the financial statements present fairly, in all material respects, the financial position of Higher Education Training Health Wellness Development Centre NPC (the Company) as at 31 March 2023, and its financial performance and cash flows for the year then ended in accordance with the International Financial Reporting Standards and the requirements of the Companies Act of South Africa.

What we have audited

Higher Education Training Health Wellness Development Centre NPC's financial statements set out on pages 11 to 30 comprise:

- the statement of financial position as at 31 March 2023;
- the statement of Profit or Loss and Other Comprehensive Income for the year then ended;
- the statement of changes in equity for the year then ended;
- the statement of cash flows for the year then ended and
- the notes to the financial statements, which include a summary of significant accounting policies.

Basis for opinion

We conducted our audit in accordance with International Standards on Auditing (ISAs). Our responsibilities under those standards are further described in the *Auditor's responsibilities for the audit of the financial statements* section of our report.

We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion.

Independence

We are independent of the Company in accordance with the Independent Regulatory Board for Auditors' *Code of Professional Conduct for Registered Auditors* (IRBA Code) and other independence requirements applicable to performing audits of financial statements in South Africa. We have fulfilled our other ethical responsibilities in accordance with the IRBA Code and in accordance with other ethical requirements applicable to performing audits in South Africa. The IRBA Code is consistent with the corresponding sections of the International Ethics Standards Board for Accountants' *International Code of Ethics for Professional Accountants (including International Independence Standards)*.

Other information

The directors are responsible for the other information. The other information comprises the information included in the document titled "Higher Education Training Health Wellness Development Centre NPC Annual Financial Statements for the year ended 31 March 2023", which includes the Audit Committee Report and the Directors' Report as required by the Companies Act of South Africa. The other information does not include the financial statements and our auditor's report thereon.

Our opinion on the financial statements does not cover the other information and we do not express an audit opinion or any form of assurance conclusion thereon.

In connection with our audit of the financial statements, our responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or our knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work we have performed, we conclude that there is a material misstatement of this other information, we are required to report that fact. We have nothing to report in this regard.

Responsibilities of the directors for the financial statements

The directors are responsible for the preparation and fair presentation of the financial statements in accordance with the International Financial Reporting Standards and the requirements of the Companies Act of South Africa, and for such internal control as the directors determine is necessary to enable the preparation of financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, the directors are responsible for assessing the Company's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless the directors either intend to liquidate the Company or to cease operations, or have no realistic alternative but to do so.

Auditor's responsibilities for the audit of the financial statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes our opinion. Reasonable assurance is a high level of assurance but is not a guarantee that an audit conducted in accordance with ISAs will always detect a material misstatement when it exists.

Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with ISAs, we exercise professional judgement and maintain professional scepticism throughout the audit. We also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for our opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Company's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by the directors.
- Conclude on the appropriateness of the directors' use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Company's ability to continue as a going concern. If we conclude that a material uncertainty exists, we are required to draw attention in our auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify our opinion. Our conclusions are based on the audit evidence obtained up to the date of our auditor's report. However, future events or conditions may cause the Company to cease to continue as a going concern.
- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

We communicate with the directors regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that we identify during our audit.

PricewaterhouseCoopers Inc.

PricewaterhouseCoopers Inc.

Director: Raj Dhanlall Registered Auditors

Waterfall City, Johannesburg, South Africa

6 October 2023

STATEMENT OF FINANCIAL POSITION

as at 31 March 2023

	Note(s)	2023 R	2022 Restated* R
ASSETS			
Non-current assets			
Property, plant and equipment	2	8 305 288	9 523 923
Current assets			
Inventories	4	3 659 878	4 396 741
Trade and other receivables	3	12 827 712	3 116 441
Cash and cash equivalents	5	38 790 771	55 662 414
		55 278 361	63 175 596
Total assets		63 583 649	72 699 519
EQUITY AND LIABILITIES			
Equity			
Retained income		6 065 743	9 651 577
Liabilities			
Current liabilities			
Trade and other payables	6	5 936 164	3 127 841
Lease liabilities	7	1 280 291	2 896 458
Deferred income	8	48 319 553	55 526 352
Provisions	9	1 981 886	1 497 291
		57 517 894	63 047 942
Total equity and liabilities		63 583 637	72 699 519

STATEMENT OF PROFIT OR LOSS AND OTHER COMPREHENSIVE INCOME

for the year ended 31 March 2023

	Note(s)	2023 R	2022 Restated* R
Revenue	10	93 322 641	79 817 157
Other operating income	11	897 385	75 000
Other operating gains (losses)	12	(3 742)	–
Other operating expenses		(97 629 731)	(77 219 650)
Operating (loss) profit		(3 413 447)	2 672 507
Financial costs	13	(172 389)	(271 974)
(Loss) profit for the year		(3 585 836)	2 400 533

STATEMENT OF CHANGES IN EQUITY

for the year ended 31 March 2023

	Retained income R	Total equity R
Restated balance at 1 April 2021	7 251 044	7 251 044
Profit for the year 2022	2 400 533	2 400 533
Restated balance at 1 April 2022	9 651 579	9 651 579
Loss for the year 2023	(3 585 836)	(3 585 836)
Balance at 31 March 2023	6 065 743	6 065 743

STATEMENT OF CASH FLOWS

for the year ended 31 March 2023

	Note(s)	2023 R	2022 Restated* R
Cash flows from operating activities			
Cash (used in)/generated from operations			
Finance Costs	14	(11 705 966)	14 474 971
Net cash from operating activities	13	(172 389)	
Cash flows from investing activities		(11 878 355)	14 474 971
Purchase of property, plant and equipment	2	(3 584 500)	(1 178 583)
Proceeds from sale of property, plant and equipment	2	207 370	
Cash flows from financing activities			
Cash repayments on lease liabilities	7&11	(1 616 167)	(1 523 405)
Total cash movement for the year		(16 871 652)	11772983
Cash and cash equivalents at the beginning of the year		55 662 414	43 889 431
Cash and cash equivalents at the end of the year	5	38 790 762	55 662 414

ACCOUNTING POLICIES

for the year ended 31 March 2023

General information

Higher Education Training Health Wellness Development Centre NPC's principal nature of activity involves carrying on public benefit activities of education, training and development on health and wellness in the higher education institutions that includes universities and TVET colleges.

The company is incorporated and domiciled in South Africa. The address of its registered office is 262 Rose Avenue, Wild Olive, Riverfalls Park, Centurion, Pretoria, 0157.

1. Basis of preparation and summary of significant accounting policies

The annual financial statements have been prepared on a going concern basis in accordance with the International Financial Reporting Standards, and the Companies Act of South Africa. The annual financial statements have been prepared on the historical cost basis.

The preparation of financial statements in conformity with International Financial Reporting Standards (IFRS) requires the use of certain critical accounting estimates. It also requires management to exercise its judgement in the process of applying the company's accounting policies..

The principal accounting policies applied in the preparation of these annual financial statements are set out below. These policies have been consistently applied to all the years presented, unless otherwise stated..

1.1 Property, plant and equipment

Definition

Property, plant and equipment are tangible assets which the company holds for its own use or for rental to others and which are expected to be used for more than one period.

Recognition

An item of property, plant and equipment is recognised as an asset when it is probable that future economic benefits associated with the item will flow to the company, and the cost of the item can be measured reliably.

Initial Measurement

Property, plant and equipment is initially measured at cost.

Cost includes costs incurred initially to acquire or construct an item of property, plant and equipment and costs incurred subsequently to add to, replace part of, or service it. If a replacement cost is recognised in the carrying amount of an item of property, plant and equipment, the carrying amount of the replaced part is derecognised.

Expenditure incurred subsequently for major services, additions to or replacements of parts of property, plant and equipment are capitalised if it is probable that future economic benefits associated with the expenditure will flow to the company and the cost can be measured reliably. Day to day servicing costs are included in profit or loss in the period in which they are incurred.

Depreciation

Property, plant and equipment is subsequently stated at cost less accumulated depreciation and any accumulated impairment losses, except for land which is stated at cost less any accumulated impairment losses.

Depreciation of an asset commences when it is available for use, and ceases at the earlier of the date that the asset is classified as held for sale, or the date that the asset is derecognised.

Each part of an item of property, plant and equipment with a cost that is significant in relation to the total cost of the item is depreciated separately.

The depreciation charge for each period is recognised in surplus or deficit unless it is included in the carrying amount of another asset. The depreciable amount of an asset shall be allocated on a systematic basis over its useful life. The depreciable amount of an asset is determined after deducting its residual value.

Residual values, useful lives and depreciation methods are reviewed at each financial year end. Where there are significant changes in the expected pattern of economic consumption of the benefits embodied in the asset, the relevant changes will be made to the residual values and depreciation rates, and the change will be accounted for as a change in the accounting estimate.

The useful lives of items of property, plant and equipment have been assessed as follows:

	Depreciation method	Average useful life
Buildings	No depreciation	
Furniture and fixtures including equipment	Straight line	10–15 years
Motor vehicles	Straight line	4 years
Office equipment	Straight line	3–4 years
Computer equipment	Straight line	3–4 years
Right of use – assets	Straight line	3–4 years
Loss for the year 2023	Straight line	3–4 years
Balance at 31 March 2023	Straight line	3–4 years

1.2 Financial instruments

Initial measurement

Financial assets

When a financial asset is recognised initially, it is measured at its fair value plus, in the case of a financial asset not at fair value through profit or loss, transaction costs that are directly attributable to the acquisition or issue of the financial asset.

Financial assets with embedded derivatives are considered in their entirety when determining whether their cash flows are solely payment of principal and interest.

Financial liabilities

Financial liabilities are recognised initially at fair value and, in the case of loans and borrowings and payables, net of directly attributable transaction costs.

The company's financial liabilities include trade and other payables, loans and borrowings including bank overdrafts, and derivative financial instruments.

Until 31 March 2023, the company classified its financial assets and financial liabilities in the following categories:

- Financial assets at fair value through profit or loss – held for trading
- Financial assets at fair value through profit or loss – designated
- Held-to-maturity investment
- Loans and receivables
- Available-for-sale financial assets
- Financial liabilities at fair value through profit or loss – held for trading
- Financial liabilities at fair value through profit or loss – designated
- Financial liabilities measured at amortised cost

The classification depended on the purpose for which the investments were acquired. Management determined the classification of its investments at initial recognition and, in the case of assets classified as held-to-maturity, re-evaluated this designation at the end of each reporting period.

The company could choose to reclassify a non-derivative trading financial asset out of the held for trading category if the financial asset was no longer held for the purpose of selling it in the near term. Financial assets other than loans and receivables were permitted to be reclassified out of the held for trading category only in rare circumstances arising from a single event that was unusual and highly unlikely to recur in the near term. In addition, the company could choose to reclassify financial assets that would meet the definition of loans and receivables out of the held for trading or available-for sale categories if the company had the intention and ability to hold these financial assets for the foreseeable future or until maturity at the date of reclassification.

Reclassifications were made at fair value as of the reclassification date. Fair value became the new cost or amortised cost as applicable, and no reversals of fair value gains or losses recorded before reclassification date were subsequently made. Effective interest rates for financial assets reclassified to loans and receivables and held-to-maturity categories were determined at the reclassification date. Further increases in estimates of cash flows adjusted effective interest rates prospectively.

Accounting policies *continued*

1. Basis of preparation and summary of significant accounting policies *continued*

1.2 Financial instruments *continued*

Initial measurement *continued*

Subsequent measurement

The measurement at initial recognition did not change on adoption of IFRS 9 – refer to description above.

Loans and receivables and held-to-maturity investments were subsequently carried at amortised cost using the effective interest method.

Available-for-sale financial assets and financial assets at fair value through profit or loss were subsequently carried at fair value. Gains or losses arising from changes in the fair value were recognised as follows:

- for financial assets at fair value through profit or loss – in profit or loss within other gains/(losses)
- for available-for-sale financial assets that are monetary securities denominated in a foreign currency – translation differences related to changes in the amortised cost of the security were recognised in profit or loss and other changes in the carrying amount were recognised in other comprehensive income
- for other monetary and non-monetary securities classified as available-for-sale – in other comprehensive income. Details on how the fair value of financial instruments is determined are disclosed in the note on fair value measurements.

When securities classified as available-for-sale were sold, the accumulated fair value adjustments recognised in other comprehensive income were reclassified to profit or loss as gains and losses from investment securities.

Impairment

The company assessed at the end of each reporting period whether there was any objective evidence that a financial asset or group of financial assets was impaired. If any such evidence existed, the extent of the impairment was determined.

Impairment losses in financial assets carried at amortised cost were recognised in profit or loss.

Impairment losses were reversed when an increase in the financial asset's recoverable amount could be related objectively to an event occurring after the impairment was recognised, subject to the restriction that the carrying amount of the financial asset at the date that the impairment was reversed could not exceed what the carrying amount would have been had the impairment not been recognised.

Impairment losses on financial assets available-for-sale was removed from equity and recognised in profit or loss. Impairment losses on equity instruments that were recognised in profit or loss were not reversed through profit or loss in a subsequent period. Reversals of impairment losses were recognised in profit or loss except for equity investments classified as available-for-sale.

Trade and other receivables

Trade receivables are measured at initial recognition at fair value plus transaction costs. They are subsequently measured at amortised cost using the effective interest rate method, less allowance for expected credit losses. For trade receivables and contract assets, a simplified approach is applied in calculating expected credit losses. Instead of tracking changes in credit risk, a loss allowance is recognised based on lifetime expected credit losses at each reporting date. A provision matrix was established that is based on the company's historical credit loss experience, adjusted for forward-looking factors specific to the debtors and the economic environment.

Trade and other receivables are classified as debt instruments and loan commitments at amortised cost.

Up to 31 March 2023, trade receivables were recognised initially at the transaction price. They were subsequently measured at amortised cost using the effective interest method, less provision for impairment. A provision for impairment of trade receivables was established when there was objective evidence that the company would not be able to collect all amounts due according to the original terms of the receivables.

Trade and other receivables were classified as loans and receivables up to 31 March 2023.

Cash and cash equivalents

Cash and cash equivalents comprise cash on hand and demand deposits, and other short-term highly liquid investments that are readily convertible to a known amount of cash and are subject to an insignificant risk of changes in value. These are initially recorded at fair value and subsequently carried at amortised cost.

Trade and other payables

Trade payables are initially measured at fair value plus direct transaction costs, and are subsequently measured at amortised cost, using the effective interest rate method.

1.3 Tax

Income Tax

Higher Health is an approved public benefit organisation in terms of section 30 of the Income Tax Act (No.58 of 1962) and the receipts and accruals of Higher Health are exempt from income tax in terms of section 10 (1) (cN) of the Act.

1.4 Leases

Leases – lessor

Definition

A lease is a contract, or part of a contract, that conveys the right to use an asset for a period of time in exchange for consideration.

Identification of a lease

At inception of a contract, it is assessed to determine whether the contract is, or contains, a lease. A contract is, or contains, a lease if the contract conveys the right to control the use of an identified asset for a period of time in exchange for consideration. If the terms and conditions of a contract are changed, it is reassessed to once again determine if the contract is still or now contains a lease.

Where a contract contains a lease component as well as a non-lease components, the consideration is allocated between the components in accordance with the requirements of revenue from contracts with customers.

Lease term

The lease term of a lease is determined as the non-cancellable period of the lease, together with the periods covered by an option to extend the lease where there is reasonable certainty that the option will be exercised by the lessee, and periods covered by an option to terminate the lease if there is reasonable certainty that the option will not be exercised by the lessee.

The assessment of the reasonable certainty of the exercising of options to extend the lease by the lessee, or not exercising of options to terminate the lease by the lessee, is reassessed upon the occurrence of either a significant event or a significant change in circumstances that is within the company's control and it affects the reasonable certainty assumptions.

The assessment of the lease term is revised if there is a change in the non-cancellable lease period.

Classification

Leases are classified as either an operating lease or a finance lease.

A lease is classified as a finance lease if it transfers substantially all the risks and rewards incidental to ownership of an underlying asset. A lease is classified as an operating lease if it does not transfer substantially all the risks and rewards incidental to ownership of an underlying asset.

Finance leases

At the commencement date, assets held under a finance lease are recognised in its statement of financial position and presented as a receivable at an amount equal to the net investment in the lease.

Accounting policies *continued*

1. Basis of preparation and summary of significant accounting policies *continued*

1.4 Leases *continued*

Leases – lessor *continued*

Initial measurement

The net investment in the lease is measured using the interest rate implicit in the lease. In the case of a sublease, if the interest rate implicit in the sublease cannot be readily determined, the discount rate used for the head lease (adjusted for any initial direct costs associated with the sublease) is used to measure the net investment in the sublease. Initial direct costs are included in the initial measurement of the net investment in the lease and reduce the amount of income recognised over the lease term.

Operating leases

Lease payments from operating leases are recognised as income on either a straight-line basis or another systematic basis. Another systematic basis is applied if that basis is more representative of the pattern in which benefit from the use of the underlying asset is diminished.

Costs, including depreciation, incurred in earning the lease income is recognised as an expense.

Initial direct costs incurred in obtaining an operating lease are added to the carrying amount of the underlying asset and those costs are recognised as an expense over the lease term on the same basis as the lease income.

Depreciation and impairment is calculated and recognised on the underlying asset in accordance with the relevant policy for the class of underlying asset.

Leases – lessee

Definition

A lease is a contract, or part of a contract, that conveys the right to use an asset for a period of time in exchange for consideration.

Identification of a lease

At inception of a contract, it is assessed to determine whether the contract is, or contains, a lease. A contract is, or contains, a lease if the contract conveys the right to control the use of an identified asset for a period of time in exchange for consideration. If the terms and conditions of a contract are changed, it is reassessed to once again determine if the contract is still or now contains a lease.

Where a contract contains a lease, each lease component with the contract is accounted for separately from the non-lease components. The consideration is then allocated to each lease component on the basis of the relative stand-alone price of the lease component and the aggregate stand-alone price of the non-lease components. The relative stand-alone price of lease and non-lease components are determined on the basis of the price the lessor, or a similar supplier, would charge an entity for that component, or a similar component, separately. If an observable stand-alone price is not readily available, an estimate of the stand-alone price is made, maximising the use of observable information in each case. All non-lease components are accounted for in accordance with whatever other policy is applicable to them.

Lease term

The lease term of a lease is determined as the non-cancellable period of the lease, together with the periods covered by an option to extend the lease where there is reasonable certainty that the option will be exercised, and periods covered by an option to terminate the lease if there is reasonable certainty that the option will not be exercised.

The assessment of the reasonable certainty of the exercising of options to extend the lease or not exercising of options to terminate the lease is reassessed upon the occurrence of either a significant event or a significant change in circumstances that is within the company's control and it affects the reasonable certainty assumptions.

The assessment of the lease term is revised if there is a change in the non-cancellable lease period.

Recognition

At inception, a right-of-use asset and a lease liability is recognised. Right-of-use assets are included in the statement of financial performance within a classification relevant to the underlying asset, and not as a separate line item.

Measurement

Right-of-use assets are initially measured at cost, comprising the following:

- the amount of the initial measurement of the lease liability;
- any lease payments made at or before the commencement date, less any lease incentives received;
- any initial direct costs incurred; and
- an estimate of costs to be incurred in dismantling and removing the underlying asset, restoring the site on which it is located or restoring the underlying asset to the condition required by the terms and conditions of the lease, unless those costs are incurred to produce inventories. The obligation for those costs are incurred either at the commencement date

or as a consequence of having used the underlying asset during a particular period. Subsequently, right-of-use assets are measured using the cost model.

Subsequently, right-of-use assets are measured using the cost model, except for investment property which is measured at fair value in accordance with the policies for investment property.

Subsequently, right-of-use assets are measured using the fair value model using the policies applicable to the relevant class of the underlying asset.

Where a lease transfers ownership of the underlying asset by the end of the lease term or if the cost of the right-of-use asset reflects a purchase option will be exercised, the right-of-use asset is depreciated from the commencement date to the end of the useful life of the underlying asset. Otherwise, the right-of-use asset is depreciated from the commencement date to the earlier of the end of the useful life of the right-of-use asset or the end of the lease term.

The company tests for impairment where there is an indication that a right-of-use asset may be impaired. An assessment of whether there is an indication of possible impairment is done at each reporting date. Where the carrying amount of a right-of-use asset is greater than the estimated recoverable amount, it is written down immediately to its recoverable amount. The resulting impairment loss is recognised immediately in surplus or deficit, except where the decrease reverses a previously recognised revaluation increase for the same asset the decrease is recognised in other comprehensive income to that extent and reduces the amount accumulated in equity under revaluation surplus, and future depreciation charges are adjusted in future periods to allocate the revised carrying amount, less its residual value, on a systematic basis over its remaining useful life.

The lease liability is initially measured at the present value of the lease payments that are not yet paid at the commencement date. Lease payments are discounted using the interest rate implicit in the lease, if the rate can be readily determined, else it is based on the company's incremental borrowing rate. The following lease payments are included where they are not paid at the commencement date:

- fixed payments, less any lease incentives receivable;
- variable lease payments that depend on an index or a rate, initially measured using the index or rate as at the commencement date;
- amounts expected to be payable under residual value guarantees;
- the exercise price of a purchase option if there is reasonably certainty that the option will be exercised; and
- payments of penalties for terminating the lease, if the lease term reflects the exercising an option to terminate the lease.

Accounting policies *continued*

1. Basis of preparation and summary of significant accounting policies *continued*

1.4 Leases *continued*

Leases – lessee *continued*

Measurement continued

Subsequently, the lease liability is measured by:

- increasing the carrying amount to reflect interest on the lease liability;
- reducing the carrying amount to reflect the lease payments made; and
- remeasuring the carrying amount to reflect any reassessment or lease modifications or to reflect revised in-substance fixed lease payments.

Interest on the lease liability in each period during the lease term is the amount that produces a constant periodic rate of interest on the remaining balance of the lease liability. The periodic rate of interest is the discount rate described above, or if applicable the revised discount rate described below.

Surplus or deficit for the year will include the interest expense on the lease liability, and the variable costs not included in the measurement of the lease liability are included in the year in which the event of condition that triggers the payment of the variable costs occurs.

1.5 Inventories

Inventory consists of medical consumables and hygiene products.

Inventories are measured at the lower of cost and estimated selling price less costs to complete and sell, on the first-in, first-out (FIFO) basis.

Net realisable value is the estimated selling price in the ordinary course of business, less estimated costs of completion and selling expenses. In general, cost is determined using a weighted average basis and includes expenditure incurred in acquiring the inventories and bringing them to their existing location and condition;

Where necessary, an impairment is made for obsolete and slow moving inventory. Damaged and expired inventory is written off to the statement of profit or loss on a quarterly basis as and when these occur.

The carrying amount of inventories is not pledged as security for any liabilities

1.6 Employee benefits

Short-term employee benefits

Compensation paid to employees for the rendering of services are recognised at the undiscounted amount paid or expected to be paid in the accounting period in which the services were rendered.

Where employees accumulate entitlement for paid absences, an expense is recognised as the additional amount that the entity expects to pay as a result of the unused entitlement that has accumulated at the end of the reporting period. In the case of non-accumulating paid absences, the expense is recognised only when the absences occur.

The expected cost of profit-sharing and bonus payments are recognised when there is a present legal or constructive obligation to make such payments as a result of past events, and a reliable estimate of the obligation can be made. A present obligation exists when there is no realistic alternative but to make the payments.

Termination benefits

A liability for termination benefit is recognised at the earlier of when the offer can no longer be withdrawn and when the related restructuring costs are recognised.

1.7 Provisions and contingencies

A provision is a liability of uncertain timing or amount. A liability is a present obligation of the entity arising from past events, the settlement of which is expected to result in an outflow from the entity of resources embodying economic benefits.

A contingent liability is:

- a possible obligation that arises from past events and whose existence will be confirmed only by the occurrence or non occurrence of one or more uncertain future events not wholly within the control of the entity; or
- a present obligation that arises from past events but is not recognised because it is not probable that an outflow of resources embodying economic benefits will be required to settle the obligation, or the amount of the obligation cannot be measured with sufficient reliability.

The amount recognised as a provision is the best estimate of the expenditure required to settle the present obligation at the end of the reporting period. Where the effect of the time value of money is material, the amount of a provision is the present value of the expenditures expected to be required to settle the obligation.

Where some or all of the expenditure required to settle a provision is expected to be reimbursed by another party, the reimbursement is recognised when it is virtually certain that reimbursement will be received when the obligation is settled. The reimbursement is treated as a separate asset. The amount recognised for the reimbursement will not exceed the amount of the provision.

Provisions are reviewed at the end of each reporting period and adjusted to reflect the current best estimate. If it is no longer probable that an outflow of resources embodying economic benefits will be required to settle the obligation, the provision is reversed.

A provision is used only for expenditures for which the provision was originally recognised. Provisions are not recognised for future operating losses.

The present obligation under an onerous contract is recognised and measured as a provision.

A constructive obligation to restructure arises only when:

- there is a detailed formal plan for the restructuring identifying at least the business or part of a business concerned, the principal locations affected, the location, function, and approximate number of employees who will be compensated for terminating their services, the expenditures that will be undertaken and when the plan will be implemented; and
- there has been raised a valid expectation in those affected that the restructuring will be carried out by starting to implement that plan or announcing its main features to those affected by it.

After initial recognition and until the liability is settled, cancelled or expires, a contingent liability is recognised in a business combination at the higher of the amount that would be recognised as a provision, and the amount initially recognised less cumulative amortisation.

Contingent assets and liabilities are not recognised, but details are disclosed in the notes to the annual financial statements.

1.8 Government grants

Government grants, including non-monetary grants at fair value, are not recognised until there is reasonable assurance that:

- the entity will comply with the conditions attaching to them; and
- the grants will be received.

Higher Health is allocated an unconditional transfer from the Department of Higher Education and Training annually and this is included in the medium-term expenditure estimate of the Department.

Accounting policies *continued*

1. Basis of preparation and summary of significant accounting policies *continued*

1.9 Revenue from contracts with customers

Revenue is income arising in the course of an entity's ordinary activities.

A contract with a customer is recognised when all of the following criteria are met:

- the contract has been approved and all parties to the contract are committed to performing their respective obligations;
- each party's rights regarding the goods or services to be transferred are identifiable;
- payment terms for the goods or services to be transferred are identifiable;
- the contract has commercial substance; and
- it is probable that the consideration in exchange for the goods or services that will be transferred will be collected.

At the inception of a contract, the goods or services promised in the contract are assessed and a performance obligation is identified for each promise to transfer to the customer either:

- a good or service that is distinct; or
- a series of distinct goods or services that are substantially the same and that have the same pattern of transfer.

Revenue is recognised when or as the performance obligation is satisfied by transferring a promised good or service to a customer. Assets are transferred when or as the customer obtains control of that asset.

1.10 Presentation Currency

The Presentation Currency is South African Rands.

1.11 Related parties

A related party is a person or entity with the ability to control or jointly control the other party, or exercise significant influence over the other party, or vice versa, or an entity that is subject to common control, or joint control. As a minimum, the following are regarded as related parties of the reporting entity:

- A person or a close member of that person's family is related to a reporting entity if that person:
 - has control or joint control of the reporting entity;
 - has significant influence over the reporting entity; or
 - is a member of the key management personnel of the reporting entity or of a parent of the reporting entity.
- An entity is related to the reporting entity if any of the following conditions apply:
 - The entity and the reporting entity are members of the same group (which means that each parent, subsidiary and fellow subsidiary is related to the others);
 - The entity is controlled or jointly controlled by a person identified as a related party;
 - The entity, or any member of a group of which it is a part, provides key management personnel services to the reporting entity or to the parent of the reporting entity.

1.12 Changes in accounting policies and disclosures

There were no new and revised pronouncements selected for adoption in the future.

NOTES TO THE ANNUAL FINANCIAL STATEMENTS

for the year ended 31 March 2023

2. Property, plant and equipment

	2023			2022		
	Cost or revaluation	Accumulated depreciation	Carrying value	Cost or revaluation	Accumulated depreciation	Carrying value
Furniture and fixtures	6 085 721	(1 399 104)	4 686 617	5 637 452	(826 143)	4 811 309
Motor vehicles	4 046 994	(2 768 508)	1 278 486	3 149 628	(1 864 877)	1 284 751
Office equipment	131 901	(50 120)	81 781	106 519	(19 700)	86 819
Computer equipment	2 611 599	(1 067 599)	1544000	1644313	(704 213)	940 100
Right of Use Assets	6 957 279	(6 242 875)	714 404	5 985 019	(3 584 075)	2 400 944
Total	19 833 494	(11 528 206)	8 305 288	16522931	(6 999 008)	9 523 923

Reconciliation of property, plant and equipment – 2023

	Opening balance	Additions	Disposals	Depreciation	Total
Furniture and fixtures	4811309	552 459	(95 425)	(581 726)	4 686 617
Motor vehicles	1 284 751	897 366		(903 631)	1 278 486
Office equipment	86 819	25 382		(30 420)	81 781
Computer equipment	940 100	1 137 033	(115 687)	(417 446)	1544000
Right of Use Assets	2 400 944	972 260		(2 658 800)	714 404
	9 523 923	3 584 500	(211 112)	(4 592 023)	8 305 288

Reconciliation of property, plant and equipment – 2022

	Opening balance	Additions through business combinations	Additions	Depreciation	Total
Furniture and fixtures	4 351 805	524 337		(64 833)	4 811 309
Motor vehicles	2 072 158			(787 407)	1 284 751
Office equipment	505 991	23 321		(442 493)	86 819
Computer equipment	627 847	630 925		(318 672)	940 100
Right of use assets	3 681 065		888 677	(2 168 798)	2 400 944
	11 238 866	1 178 583	888 677	(3 782 203)	9 523 923

	2023 R	2022 R
3. Trade and other receivables		
Financial instruments:		
Trade receivables	715 246	169 357
Other receivables HW SETA & HW SETA CET	4 713 966	
Sundry Debtors		7 047
Non-financial instruments:		
VAT	6 946 888	2 488 198
Prepaid expenses	451 612	451 839
Total trade and other receivables	12 827 712	3116441
4. Inventories		
Inventories	3 659 878	4 396 741

It is expected that all consumable inventory will be utilised in the entity's operations during the next 12 months.

Notes to the annual financial statements *continued*

	2023 R	2022 R
5. Cash and cash equivalents		
Cash and cash equivalents consist of:		
Bank balances	21 050 616	39 004 506
Other cash and cash equivalents – Investec	17 740 155	16 657 908
	38 790 771	55 662 414
6. Trade and other payables		
Financial instruments		
Trade payables	3 453 402	1 360 273
Accrued liabilities	179 5874	679 511
Other payables	686 888	1 088 057
	5 936 164	3 127 841
Accrued liabilities, other payables and trade payables are carried at amortised cost, and their carrying values approximate their fair values. Trade payables are settled on terms negotiated with the respective suppliers which is usually 30 days of invoice date.		
7. Finance Lease Liabilities		
Lease liabilities		
The maturity analysis of lease liabilities is as follows:		
Within one year	1 032 513	2 366 736
Two to five years	247 778	529 722
Present Value of minimum lease payments	1 280 291	2 896 458
It is company policy to lease certain assets under finance leases. The entity leases buildings and photocopiers. The average lease term is 36 months and the average effective borrowing rate between 8% and 9%. Rental on buildings increase between 8% and 9% every 12 months until the end of the lease term. Interest rates are linked to prime at the contract date. There is no escalation on the lease of photocopiers.		
Contingent payments on the lease of the photocopiers is limited to the number of photocopies made. There is no contingent rent on the rental of the property itself apart from additional charges incurred relating to usage of certain services such as diesel for the office generators or access cards.		
A new lease was entered into in the 2023 period for the lease of office premises in Belville and for the lease of all photocopiers.		
Lease payments recognised as an expense	1085449	1 251 339

	2023 R	2022 R
8. Deferred income		
Heading		
DHET Funding	17 740 155	16 657 908
CET	10 682 172	11 114 795
NSF	6 331 896	314 120
HEAID Membership	2 720 192	2 720 192
TVETS Membership	1469662	1469662
Global Fund NACOSA	6 380 173	4 900 960
HW SETA		1 230 781
DHET Treasury allocation	922 455	12 710 432
Atlantic Philanthropie	2 072 848	1963547
Sage Welcome Trust		1 023 662
AFSA reactors project		(97 430)
HW SETACET		1 505 096
Old Mutual		12 627
	48319553	55 526 352
Split between non-current and current portions		
Current liabilities	48 319 533	55 526 352

9. Provisions

Reconciliation of provisions – 2023

	Opening balance	Additions	Total
Leave Pay Provision	1 069 813	265 103	1 334 916
Performance Bonus Provision	427 478	219 492	646 970
	1 497 291	484 595	1 981 886

Reconciliation of provisions – 2022

	Opening balance	Additions	Total
Leave Pay Provision	1 658 792	(588 979)	1 069 813
Performance Bonus Provision	649 576	(222 098)	427 478
	2 308 368	(811 077)	1 497 291

	2023 R	2022 R
10. Revenue		
Revenue from contracts with customers		
Grants received	93 322 641	79 817 157
11. Other income		
Sundry Income – Nacosa & Donation Income	897 385	75 000
12. Other operating gains (losses)		
Gains (losses) on disposals, scrappings and settlements		
Property, plant and equipment	(3 742)	
13. Finance costs		
Operating lease liabilities	172 389	

Notes to the annual financial statements *continued*

	2023 R	2022 R
14. Cash (used in)/generated from operations		
Loss/(Profit) before taxation	(3 620 336)	2 400 533
Adjustments for non-cash items:		
Depreciation and amortisation	4 592 023	3 782 203
Losses on sale of assets and liabilities	3 742	
Movements in provisions	484 595	(811 077)
Other adjustments for non-cash items – ROU Assets	–	(888 676)
Adjust for items which are presented separately:		
Finance costs	172 389	271 974
Changes in working capital:		
(Increase) decrease in inventories	736 863	(439 674)
(Increase) decrease in trade and other receivables	(9 676 771)	(1 768 651)
Increase (decrease) in trade and other payables	2 808 328	917 422
Increase (decrease) in deferred income	(7 206 799)	14 967 984
	(11 705 966)	14 474 971

15. Related parties

Relationships

Members of key management

Dr Ramneek Ahluwalia (Chief Executive Officer) Mr Prakash Narismulu
(Director: Finance and Corporate Services): Appointed 1 March 2022, resigned
effective 31 December 2022

Mr Tendai Munyoro (Director Finance and Corporate Services): Appointed
1 February 2023

Ms Thobekile Finger (Director of Programmes) Ms Nthuseng Celina Mphahlele
(Director of Operations): Appointed 1 April 2023

Compensation to directors and other key management

Short-term employee benefits

6 738 144 5 119 855

16. Directors' and prescribed officer's emoluments Executive

Executive

2023

	Directors emoluments	Bonuses and performance related payments	Backpay	Other allowances	Total
Directors' emoluments					
Services as director or prescribed officer					
Dr (Prof) R Ahluwalia	2 500 547	265 857	197 838	455 557	3 419 799
	2 500 547	265 857	197 838	455 557	3 419 799

2022

	Directors emoluments	Bonuses and performance related payments	Other benefits	Total
Directors' emoluments				
Services as director or prescribed officer				
Dr (Prof) R Ahluwalia (CEO)	2 094 899	222 771	352 127	2 669 797
	2 094 899	222 771	352 127	2 669 797

Service contracts

One Director, the Higher Education Training Health Wellness Development Centre NPC Chief Executive Officer, is subject to a written employment contract agreement. The employment agreement regulate duties, remuneration, allowances, restraints, leave and notice periods of this executive. The service contract does not exceed five years.

Prescribed officers

2023

	Emoluments	Other benefits	Total
Mr P Narismulu Director: Finance and Corporate Services (Appointed 1 March 2022, resigned effective 31 December 2022)	1 090 386	218 933	1 309 319
Mr Tendai Munyoro Director Finance and Corporate Services (Appointed 1 February 2023)	262 275	19 133	281 408
Mrs T Finger	1439534	288 083	1727617
	2 792 195	526 149	3 318 344

2022

	Emoluments	Other benefits	Total
Mr M Takundwa Director: Finance and Corporate Services (Resigned 2022)	437 328	10 407	447 735
Mr ET Chetty Acting Finance Director (Appointed 1 July 2021; Deceased 11 January 2022)	676 636	25 396	702 032
Mr P Narismulu Director: Finance and Corporate Services (Appointed 1 March 2022)	121 154	11 000	132 154
Mrs T Finger	980 346	184 792	1 165 138
	2 215 464	231 595	2 447 059

Notes to the annual financial statements *continued*

17. Prior period corrections: Reclassifications

	2022
Statement of financial Performance	
Cost of sales previously stated	158 880
Reclassification to Operating expense (Consulting & Professional fees)	(158 880)
	–
Statement of financial Performance	
Operating expense previously stated	77 060 770
Reclassification cost of sales	158 880
	77 219 650
Furniture & Fittings: Cost	
Previously reported	1 052 636
Reclassification: Correction of vending machines incorrectly reflected as office equipment instead of furniture & Fixtures	4 584 816
Restated amount	5 637 452
Furniture & Fittings: Accumulated Depreciation	
Previously reported	(160 277)
Reclassification: Correction of vending machines incorrectly reflected as office equipment instead of furniture & Fixtures	(665 866)
Restated amount	(826 143)
Office Equipment: Cost	
Previously reported	4 691 335
Correction of vending machines incorrectly reflected as office equipment instead of furniture & Fixtures	(458 481)
Restated amount	106 519
Office Equipment: Accumulated Depreciation	
Previously reported	(685 566)
Reclassification: Correction of vending machines incorrectly reflected as office equipment instead of furniture & Fixtures	665 866
Restated amount	(19 700)

18. Financial instruments and risk management Financial risk management

Credit risk

Credit risk is the risk of financial loss to the Entity if a customer or counterpart to a financial instrument fails to meet its contractual obligations, and arises principally from the Entity's receivables. The Entity's exposure to credit risk is influenced mainly by the individual characteristics of each customer and risk is independently assessed based on the characteristics of the receivables. The following financial statement line items are affected by credit risk:

	2023 R	2022 R
Trade and Other Receivables	12 827 712	3116441

Liquidity risk

Liquidity risk is the risk that the Entity will not be able to meet its financial obligations as they fall due. The

Entity's approach to managing liquidity is to ensure, as far as possible, that it will always have sufficient liquidity to meet its liabilities when due, under both normal and stressed conditions, without incurring unacceptable losses or risking damage to the Entity's reputation.

The following are the contractual maturities of financial liabilities.

	2023
Current liabilities	
Trade and other payables	5 936 164
Lease liabilities	1 280 291
	7216455
	2022
Current liabilities	
Trade and other payables	3 127 836
Lease liabilities	2 896 458
	6 024 294

Interest rate risk

As the Entity has interest bearing assets with interest rates related to market rates, the Entity's income and operating cash flows from interest are dependent on changes in market interest rates. The following financial statement line items are affected by interest rates.

	Note(s)	2023 R	2022 R
Interest rate risk			
Cash and Cash Equivalents		44 025 587	55 662 413

DETAILED INCOME STATEMENT

for the year ended 31 March 2023

	Note(s)	2023 R	2022 Restated* R
Revenue			
Grants received		93 322 641	79 817 157
Other income			
Sundry income: Nacosa/Donation Income		897 385	75 000
Operating expenses			
Accounting fees		(132 353)	(636 160)
Auditors remuneration – external auditors		(1 733 149)	(679 334)
Bad debts		(7 047)	
Cleaning		–	(59 356)
Computer expenses		(996 538)	(847 376)
Consulting fees		(16 360 843)	(14 438 739)
Depreciation		(4 592 023)	(3 782 203)
Donations		(541 629)	(29 794)
Employee costs		(45 200 309)	(34 987 165)
Freight & courier		(740 666)	(256 350)
General expenses		(931)	
Grant assets expensed		(96 980)	(131 709)
Grants to universities		(3 970 210)	(4 351 272)
Insurance		(852 292)	(734 228)
Legal expenses		(85 930)	(388 783)
Municipal expenses		(437 750)	(281 669)
Printing and stationery		(1 539 308)	(1 826 382)
Profit (loss) on sale of assets and liabilities		(3 742)	
Promotional and IEC Material		(1 892 345)	(2 102 497)
Rent paid		(1 136 127)	(1 329 840)
Repairs and maintenance		(291 084)	(361 680)
Research and development costs		(34 712)	
Security		(86 036)	(59 975)
Staff welfare		(264 759)	(151 942)
Subscriptions		(446 294)	(98 196)
Telephone and fax		(1 836 373)	(1 618 775)
Travel – local		(13 562 438)	(7 597 745)
Workshops and Capacity Development		(791 605)	(468 480)
		(97 633 473)	(77 219 650)
Operating surplus		(3 413 447)	2 672 507
Finance costs	13	(172 389)	(271 974)
(Loss) profit for the year		(3 585 836)	2 400 533

The supplementary information presented does not form part of the annual financial statements and is unaudited.

* See Note 17

The supplementary information presented does not form part of the annual financial statements and is unaudited.



HIGHER HEALTH

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